An open label, multicenter, phase II study of Elranatamab as single agent for the treatment of relapsed or refractory myeloma in patients previously exposed to three-drug classes

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This study protocol was subjected to critical review and has been approved by the Sponsor. The information it contains is consistent with the current risk/benefit evaluation of the investigational product as well as with the moral, ethical, and scientific principles governing clinical research as set out in the Declaration of Helsinki and the guidelines on Good Clinical Practice.

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I have read this study protocol, including all appendices. By signing this protocol, I agree to supervise and conduct the clinical study, following approval by an Independent Ethics Committee (IEC), in accordance with the study protocol, the current International Conference on Harmonisation (ICH-E6-R2) Guideline for Good Clinical Practice (GCP), and applicable regulatory requirements. I will ensure that all personnel involved in the study under my direction will be informed about the contents of this study protocol and will receive all necessary instructions for performing the study according to the study protocol.

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1. ABBREVIATIONS

Table 1 Abbreviations

Abbreviation	Full Terminology
AE	Adverse Event
AESI	Adverse Event Of Special Interest
ALT	Alanine Transaminase
ANC	Absolute Neutrophil Count
ASCT	Autologous Stem Cell Transplant
AST	Aspartate Transaminase
BM	Bone Marrow
BP	Biological Progression
CA	Competent Authority
CCI	Charlson Comorbidity Index (Cci)
CI	Confidence Interval
ClCr	Creatinine Clearance
CONSORT	Consolidated Standards Of Reporting Trials
COPD	Chronic Obstructive Pulmonary Disease
CR	Complete Response
CRA	Contract Research Associate
CRF	Case Report Form
CRO	Contract Research Organization
CRP	C-Reactive Protein
CRS	Cytokine Release Syndrome
CT	Computed Tomography
CTCAE	Common Terminology Criteria For Classification Of Adverse Events
DL (-1, 1, 2, 3)	Dose Level (-1, 1, 2, 3)
DSMC	Data Safety Monitoring Committee
DNA	Deoxyribonucleic Acid
DVT	Deep Vein Thrombosis
EC	European Commission
ECG	Electrocardiogram

Abbreviation	Full Terminology
ECHO	Echocardiogram
ECOG	Eastern Cooperative Oncology Group
eCRF	Electronic Case Report Form
EDC	Electronic Data Capture
EDTA	Ethylenediamine tetra Acetic acid
ELISA	Enzyme-Linked Immunosorbent Assay
EMA	European Medicines Agency
EOT	End Of Treatment
EPO	Erythropoietin
EU	European Union
FCBP	Female Of Childbearing Potential
FNCBP	Female Not Of Childbearing Potential
FEV1	Forced Expiratory Volume In The First Minute
FISH	Fluorescence In Situ Hybridization
FTIH	First Time In Human
GAH	Geriatric Assessment In Hematology
GBS	Guillain-Barré Syndrome
GCP	Good Clinical Practice
G-CSF	Granulocyte Colony-Stimulating Factor
GVHD	Graft Vs. Host Disease
HBV	Hepatitis B Virus
HCV	Hepatitis C Virus
HIV	Human Immunodeficiency Virus
HR	Hazard Ratio
HSC	Hematopoietic Stem Cells
IAT	Indirect Antiglobulin Test
ICANS	Immune Effector Cell-Associated Neurotoxicity Syndrome
ICD	Immunogenic Cell Death
ICF	Informed Consent Form
ICH	International Conference On Harmonization
IDMC	Independent Data Monitoring Committee

Abbreviation	Full Terminology
IEC	Independent Ethic Committee
IF	Immunofixation
Ig	Immunoglobulin
IMP	Investigational Medicinal Product
IMWG	International myeloma working Group
IP	Investigational Product
IPAL	Investigational Product Accountability Log
IRR	Infusion-Related Reaction
IRRs	Infusion-Related Reactions
ISS	International Staging System
ITT	Intention-To-Treat
IV	Intravenous
Kd	Carfilzomib And Dexamethasone
KRd	Carfilzomib, Lenalidomide And Dexamethasone
LDH	Lactate Dehydrogenase
LPLV	Last Patient Last Visit
LVEF	Left Ventricle Ejection Fraction
MC	Monoclonal Component
MDS	Myelodysplastic Syndromes
MECL1	Multicatalytic Endopeptidase Complex-Like 1
MedDRA	Medical Dictionary For Regulatory Activities
MFC	Multiparametric flow Cytometry
MGUS	Monoclonal Gammopathy Of Undetermined Significance
MM	Multiple Myeloma
MoAbs	Monoclonal Antibodies
MP	Melphalan And Prednisone
MPT	Melphalan And Prednisone And Thalidomide
MRD	Minimal Residual Disease
MRI	Magnetic Resonance Imaging
MTD	Maximum Tolerated Dose
MUGA	Multiple-Gated Acquisition Scan

Abbreviation	Full Terminology
NDMM	Newly-Diagnosed Multiple Myeloma
NF-kB	Nuclear Factor Kappa
NGF	Next Generation Flow Cytometry
NK	Natural Killer
NYHA	New York Heart Association
ORR	Overall Response Rate
OS	Overall Survival
PC	Plasmatic Cells
PCR	Polymerase Chain Reaction
PD	Progressive Disease
PET	Positron Emission Tomography
PET-TC	Positron Emission Tomography And Computed Tomography
PFS	Progression-Free Survival
PFT	Pulmonary Function Test
P-gp	P-Glycoprotein
PI	Proteasome Inhibitors
PIL	Patient Information Leaflet
PO	Per Os (Oral)
POEMS	Polyneuropathy, Organomegaly, Endocrinopathy, Monoclonal Plasma Proliferative Disorder, Skin Changes
PR	Partial Response
QC	Quality Control
QoL	Quality Of Life
REB	Research ethics Board
RBC	Red Blood Cell
Rd	Lenalidomide And Low Dose Of Dexamethasone
RD	Lenalidomide And Standard Dose Of Dexamethasone
Rh	Rhesus Factor
R-ISS	Revised International Staging System
RNA	Ribonucleic Acid
RNA-Seq	Rna Sequencing
RP2D	Recommended Phase 2 Dose

Abbreviation	Full Terminology
RRMM	Relapsed Refractory Multiple Myeloma
SAE	Serious Adverse Event
SC	Subcutaneous
sCR	Stringent Complete Response
SCT	Stem cell Transplantation
SD	Stable Disease
sFLC	Serum Free Light Chains
SoA	Schedule Of Activities
SOC	Standard Of Care
SOP	Standard Operating Procedure
SP	Symptomatic Progression
SPC	Summary Of Product Characteristics
SPEP	Serum Protein Electrophoresis
SPM	Second Primary Malignancies
SRSD	Single Reference Safety Document
TEAE	Treatment-Emergent Adverse Events
TTBR	Time To Best Response
TTP	Time To Progression
TTR	Time To Response
TNT	Time To Next Treatment
ULN	Upper Limit Of Normal
UPEP	Urine Protein Electrophoresis
VGPR	Very Good Partial Response
VMP	Bortezomib-Melphalan And Prednisone
VTP	Bortezomib, Thalidomide, And Prednisone
WBC	White Blood Cell
WBLDCT	Whole Body Low Dose Computed Tomography
WOCBP	Women of childbearing potential

2. INTRODUCTION

2.1. Study Design

This is a Phase II, open-label, single-arm, multicenter study to evaluate the efficacy and safety of elranatamab monotherapy at the dose of 76 mg subcutaneously in participants with RRMM after one or two prior lines of therapy who have received prior treatment with immunomodulatory drugs, protease inhibitors, and anti-CD38 therapy and were refractory to the last line of therapy, defined as progression while receiving treatment or in the first 60 days after the last dose of treatment.

The study consists of a screening/baseline period, a treatment period, and a posttreatment follow-up period.

The study includes a periodic review of safety data, that will be independently analyzed by the Data Safety Independent Committee (DSMC) and will recommend how to proceed with the study.

2.2. Milestones

Milestone	Planned Date
First Patient Inclusion	3Q2023
Last Patient Inclusion	1Q2025
Last Patient Last Visit (LPLV)	3Q2029

2.3. Frontline treatment

Treatment with elranatamab will be initiated using a 2-step-up priming regimen: the initial doses of elranatamab will be 12 mg (Cycle 1 Day 1) and 32 mg (Cycle1 Day 4). Participants should be hospitalized and monitored for toxicity (especially CRS/ICANS) for at least 2 days (~48 hours) beginning on Cycle 1 Day 1, and for 1 day (~24 hours) for Cycle1 Day 4. The dose of elranatamab should be increased to 76 mg on Cycle 1 Day 8 as long as the participant meets the redosing criteria or deferred until the criteria are met.

The scheme of administration includes weekly administrations for at least six 4-weeks cycles and, if patients have achieved at least PR (or better) persisting for at least 2 months, the dose interval should be changed from weekly to every other week.

Treatment will be scheduled with a response-adapted duration and patients achieving undetectable measurable residual disease and maintained for 12 months will stop therapy. After stopping therapy, and if the patient is in sustained undetectable measurable residual disease for at least 12 months, it would be possible to re-start treatment with elranatamab in case the measurable residual disease will be detectable or relapse from CR will occur. Patients who will not achieve undetectable measurable residual disease sustained for 12 months will receive continuous treatment until progressive disease. In both situations, the occurrence of unacceptable toxicity might result into the treatment discontinuation

follow-up

2.4. Study objectives and endpoints

Objectives	Endpoints
Primary	
To evaluate the clinical efficacy of elranatamab in patients with relapsed/refractory multiple myeloma.	- Rate of Undetectable Measurable Residual Disease at 6 and 12 months as per International Myeloma Working Group (IMWG) criteria evaluated by the investigators
Secondary	
To evaluate the clinical efficacy of elranatamab in patients with relapsed/refractory multiple myeloma using additional efficacy measures	- Sustained undetectable Measurable Residual Disease annually rate as per International Myeloma Working Group (IMWG) criteria evaluated by the investigators - Overall response rate and the different responses categories according to the IMWG criteria as evaluated by investigator - Duration of response - Time to first and best response - Progression-free survival - Overall survival - PFS and OS in those patients who stop treatment because of sustained MRD-ve for 12 months.
To evaluate the safety of elranatamab in patients with relapsed/refractory multiple myeloma.	-Adverse events (AEs) and serious adverse events (SAEs) collected according to standard clinical and laboratory tests (hematology and chemistry, physical examination, vital sign measurements, and diagnostic tests) -Incidence and severity of Cytokine Release Syndrome (CRS) and Immune effector cell associated neurotoxicity syndrome (ICANS) according to the American Society for Transplantation and Cellular Therapy (ASTCT) criteria -Incidence and severity of other neurotoxicities. -Incidence of cytopenias and infections

follow-up

Biological studies	
To determine biomarkers predictors of response	- Circulating Tumor Cells at baseline to evaluate its prognostic value - Measurement of serum BCMA levels and its correlation with response - Gene expression techniques (cytogenetic abnormalities and RNA sequencing and single cell sequencing studies) coupled with intelligent clinical and molecular data analysis (i.e. machine learning) to identify factors that could predict response to elranatamab.
To determine additional efficacy measures with alternative methods	- Mass spectrometry to complement the evaluation of the response by the conventional techniques like electrophoresis, immunofixation and next generation flow
To characterize patients' immune system[1]	- Identification and characterization of T, B, NK, monocytes and normal/clonal PC (including BCMA expression) - Quantification of soluble factors: IL6, IL2, IFNγ and TNFα
To define genomic determinants of response/resistance	- Fluorescence-activated cell sorting (FACS) sorting of T and Pathological Plasma Cells at inclusion and at PD, and of T cells after the first dose

[1] The studies to characterize the pts immune system will be performed at inclusion (to know the baseline, which will likely influence treatment responses and that is probably induced by the previous treatment), after the first dose (also coincident with the peak in cytokine levels), after the 3rd dose (coincident with the median time to response) and at progression (to define immune mechanisms of resistance).

3. BACKGROUND AND SIGNIFICANCE

3.1. Rationale for Current Study

Multiple myeloma (MM) is a blood cancer of monoclonal plasma cells that accumulate in the bone marrow and produce M-protein (also known as monoclonal immunoglobulin or paraprotein)[1]. Multiple myeloma accounts for 1-1.8% of neoplastic diseases, and is the second most common hematological malignancy, with an incidence in Europe of 4.5–6 per 100,000 per year and a median age of about 70 years at the initial diagnosis[1,2]. Multiple myeloma is complicated by organ dysfunction: hypercalcaemia, renal insufficiency, anemia, and bone destruction (known as the CRAB criteria) which together with the presence of at least 2 of the biomarkers of malignancy (i.e. clonal bone marrow plasma cells of at least 60%, ratio of involved versus uninvolved serum free light chain (FLC) ratio of at least 100, or more than 1 focal lesion on MRI) are indicative of the need of active therapy.

The use of novel agents such as proteasome inhibitors (PIs; bortezomib, carfilzomib, ixazomib), immunomodulatory drugs (IMiDs, thalidomide, lenalidomide, pomalidomide), and monoclonal antibodies (elotuzumab, daratumumab, isatuximab) in combination with steroids (dexamethasone, prednisone) and also high-dose therapy and autologous stem-cell transplantation (ASCT), has markedly improved outcomes in patients with multiple myeloma[1].

The most recent EHA/ESMO guidelines on the management of MM recognize those 3 main drug classes as the pillars of the treatment of MM patients[2]. Thus, EHA-ESMO guidelines recommend as front-line therapy of MM several combinations of these three drug classes with steroids usually as a triplet regimens (e.g. bortezomib/lenalidomide/dexamethasone [VRd], bortezomib/thalidomide/dexamethasone [VTD], daratumumab/lenalidomide/dexamethasone [DaraRd]), but sometimes as a doblet (lenalidomide/dexamethasone [Rd]) o quadruplet regimens (daratumumab/bortezomib/thalidomide/dexamethasone [DaraVTD]) regimens; some recommended regimens include an alkylating drug (i.e. melphalan, cyclophosphamide) instead of an IMiD (e.g. bortezomib/cyclophosphamide/dexamethasone [VCD], bortezomib/melphalan/prednisone [VMP], daratumumab/bortezomib/melphalan/prednisone [DaraVMP])[2].

Unfortunately, most patients with MM experience numerous relapses of their disease, with periods of remission increasingly shorter[1]. The optimal sequence and choice of agents for relapsed MM is not so well established and will depend on several factors such as patient characteristics (e.g. age, frailty, performance status), disease characteristics (e.g. cytogenetic risk), patient preference, availability, but mostly will depend on the type and response to previous therapies[1-3]. EHA-ESMO treatment algorithm for second-line treatment is based on whether the initial treatment was VRd, DaraRd,

DaraVMP/DaraVTD, and the sensitivity to lenalidomide and/or bortezomib, and the options are also a myriad of combinations, usually as triplets, of drugs from those three drug classes with steroids[2]. The International Myeloma Working Group has also recently issued recommendations for the treatment of relapsed or refractory MM[3].

Overall survival (OS) becomes shorter as the number of refractoriness to the drug classes increases. In 275 patients who were refractory to monoclonal antibodies, Ghandy et al reported 148 (54%) were “triple-refractory and quadruple-refractory” and 75 patients (25%) were “penta-refractory” and they found a median OS of 11.2 months (95% C.I. 5.4–17.1) for “not triple-refractory” group, 9.2 months (95% C.I. 7.1–11.2) for “triple-and quadruple-refractory” and 5.6 months (95% C.I. 3.5–7.8) for “penta-refractory” groups.

This is consistent with the Spanish experience and triple-class refractory MM is associated with a very severe disease, very short life-expectancy and many and relevant unmet needs[4].

In patients with MM who are triple-class refractory, treatment options are very limited. Only two drugs have been approved by the EMA in this setting: Selinexor (an oral selective exportin 1 inhibitor) in combination with dexamethasone[5], and belantamab mafodotin (an antibody-drug conjugate that targets BCMA) as monotherapy[6]; in addition, Ide-Cel, Cilta-Cel and teclistamab-cqy have also received the approval by the EMA in this population. EHA-ESMO guideline recommends these options in triple-class refractory patients. Immunotherapy strategies targeting BCMA or other antigens on the surface of myeloma cells, including bispecific T-cell engagers (BiTEs) and chimeric antigen receptor T (CAR-T) cells, are under clinical investigation in patients with relapsed or refractory MM [2]. Results from systematic reviews indicate that anti-BCMA CAR-T therapy are efficacious and safe in patients with relapse or refractory multiple myeloma[7,8]. Bispecific molecules, including bispecific antibodies (BisAbs) and bispecific T-cell engagers (BiTEs) have shown promising results, even in heavily pretreated patients with relapse or refractory MM, although BiTEs are not anymore in development in myeloma[9]. Among CAR-T therapies, Ide-Cel and Cilta-Cel have received the approval of the FDA and EMA for the treatment of adult patients with relapsed or refractory multiple myeloma after four or more prior lines of therapy, including a proteasome inhibitor (PI), an immunomodulatory agent (IMiD), and an anti-CD38 monoclonal antibody[10-13]. The approval of Ide-Cel (Idecabtagene vicleucel) was based on the results of the pivotal phase 2 KarMMa trial (NCT03361748), where among the patients evaluable for efficacy, the objective response rate achieved with ide-cel was 73% with stringent CRs (sCR) in 33%[14]. Cilta-cel (ciltacabtagene autoleucel) was also approved based on findings from the CARTITUDE-1 trial, in which cilta-cel led to

an objective response rate (ORR) of 97% (95% CI 91·2–99·4) in this patient population; additionally, the stringent complete response (sCR) rate was 67%[15]. In the case of the BiTEs, teclistamab-cqy, it has been recently approved by the FDA and EMA for the same indication[16,17]. The approval of teclistamab-cqy was based on the results of the MajesTEC-1 trial that showed an ORR of 63%[18]A antibody drug conjugate, belantamab mafodotin, was approved in August 2020 but FDA withheld its approval in November 2022 because the randomized phase 3 study did not confirm its superiority over pomalidomide-dexamethasone[19],

In this situation and according to the EHA/ESMO recommendations, the group of patients triple exposed is more frequent and earlier on, after just one or two prior lines of therapy and it seems appropriate to evaluate immunotherapy strategies targeting BCMA in this situation. In addition, the Spanish Myeloma Group has finalized the recruitment of a phase 3 trial in NDMM fit patients that have received the three main drug classes as part of the first line of therapy remarking how the TCE/TCR population is also a real population in our country.

3.2. Background of Investigational Product

B-cell maturation antigen (BCMA also referred to as TNFRSF17 or CD269) is a member of the tumor necrosis factor (TNF) receptor superfamily and regulates a variety of cellular functions. BCMA is expressed on malignant plasma cells in all MM patients[20,21]. The restricted expression profile of BCMA in normal tissue, combined with its up-regulation and recognized survival function in MM[21-23] makes BCMA an attractive target for a therapeutic antibody with direct cell killing activity and with minimal off target effects[20]. BCMA has been validated as a therapeutic target in MM[20]. Currently, there are three common treatment modalities under clinical investigation targeting BCMA in the treatment of MM: bispecific antibody constructs, antibody–drug conjugates, and chimeric antigen receptor (CAR)-modified T-cell therapy[24]. Among the formers, elranatamab (PF-06863135) is a humanized bispecific IgG mAb consisting of anti-CD3 and anti-BCMA-targeting arms paired through hinge-mutation technology within an IgG2a backbone[25].

In vitro, elranatamab has been shown to induce cytokine release by human T-cells and to redirect patient T-cells to lyse tumor cells from MM patients in a concentration-dependent manner. Elranatamab also demonstrated robust anti-tumor activity in vivo following a single dose in three different orthotopic human MM models established in immunodeficient mice engrafted with human T-cells, and greater potency was correlated with higher BCMA expression levels[26,27]. In another orthotopic tumor model with low BCMA expression levels, a second dosing of elranatamab was found to delay tumor progression. As part of a secondary pharmacology assessment, elranatamab induced

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cytokine release in human whole blood, which was expected due to the presence of BCMA-expressing target cells, confirming the mechanism of action. Two 1-month GLP toxicology studies in cynomolgus monkeys using IV and SC routes of administration showed mechanism-based effects, including increased T-cell activation, increased serum cytokine concentrations and microscopic findings in the secondary lymphoid tissues. Other changes noted related to the primary pharmacology included transient decreases in circulating lymphocytes and reductions in serum globulins.

To date, preliminary results of the phase 1 study of elranatamab, MagnetisMM-1, and a phase 2 study, MagnetisMM-3, have been reported.

MagnetisMM-1 (NCT03269136) is a Phase 1 study of elranatamab (PF-06863135), a humanized bispecific molecule that targets BCMA expressed on multiple myeloma (MM) and engages CD3 on T cells, for patients (pts) with relapsed or refractory MM (RRMM). There were different subcutaneous (SC) cohorts; dose escalation (Part 1), monotherapy with priming (Part 1.1), lenalidomide (LEN) combination (Part 1C), pomalidomide (POM) combination (Part 1D), and monotherapy expansion with priming (Part 2A)[28].

In Part 1, patients received elranatamab at 80, 130, 215, 360, 600, and 1000 µg/kg subcutaneously weekly guided by a modified toxicity probability interval method. For monotherapy at the recommended Phase 2 dose (RP2D; Parts 1.1 and 2A), a single priming dose (600 µg/kg or equivalent fixed dose of 44 mg) was followed one week later by the full dose (1000 µg/kg or equivalent fixed dose of 76 mg) weekly (Q1W) or every 2 weeks (Q2W) thereafter[28].

As of 22 June 2022, a total of 55 pts received single-agent elranatamab SC at a dose ≥215µg/kg. Median age was 64 (range 42-80) years, and 27% of pts were Black/African American or Asian. Median number of prior regimens was 5 (range 2-14), 91% were triple-class refractory, 69% had prior stem cell transplantation, 29% had high cytogenetic risk, and 24% received prior BCMA-targeted therapy[29].

The most common TEAEs regardless of causality included CRS, neutropenia, anemia, injection site reaction, and lymphopenia. With pre-medication and a single priming dose (600µg/kg or 44mg), the overall incidence of CRS at the recommended dose (1000µg/kg or 76mg) was 67% and limited to Grade 1 (33%) or Grade 2 (33%), with no events Grade 3 or higher. Elranatamab exposure was dose dependent. Cytokine increases occurred with the first dose and were reduced by pre-medication. Soluble BCMA decreased with disease response, elranatamab therapy was associated with increased peripheral T-cell proliferation, and median time to response was 36 days (range 7-262).

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With a median follow-up of 12.0 months (range 0.3-29.0) and including only IMWG confirmed responses, the objective response rate (ORR) was 64% (95% CI 50-75%) with 56% of pts (31/55) achieving very good partial response (VGPR) or better and 38% of pts (21/55) achieving complete response (CR) or better. Among 13 pts with prior BCMA-directed therapy (antibody drug conjugate, chimeric antigen receptor T-cell therapy, or both), 54% (7/13) achieved response including 46% (6/13) with VGPR or better. For responders (n=35), the probability of being event-free at 12 months was 59% (95% CI 39-74%) and the Kaplan-Meier estimate for median duration of response was 17.1 months (95% CI 10.6-NE).

Single-agent elranatamab induced durable clinical and molecular responses for patients with relapsed or refractory MM, and 100% (12/12) of evaluable pts with confirmed CR or better achieved MRD negativity at a sensitivity of 1×10^{-5} including 2 pts with MRD negativity and ongoing stringent CR beyond 2 years.

Together with emerging data from pivotal studies MagnetisMM-3 (NCT0469359) and MagnetisMM-5 (NCT05020236), these results support further development of elranatamab for patients with MM[29].

MagnetisMM-3 (NCT04649359) is an open-label, multicenter, non-randomized, phase 2 study to evaluate the safety and efficacy of elranatamab monotherapy in patients (pts) with relapsed/refractory MM (RRMM). Patients were assigned to 1 of 2 independent, parallel cohorts: those naïve to BCMA-directed therapies (Cohort A) and those with previous exposure to BCMA-directed antibody-drug conjugates or CAR-T cells (Cohort B)[30]. Patients are refractory to at least 1 proteasome inhibitor, 1 immunomodulatory drug, and 1 anti-CD38 antibody and received subcutaneous elranatamab 76 mg QW on a 28-day cycle with a 2-step-up priming dose regimen (12 mg and 32 mg) administered during the first week[31].

A total of 123 pts were enrolled and treated with elranatamab in Cohort A. Median age was 68.0 years (range, 36−89), 55.3% were male, 58.5% were White, 13.0% were Asian and 7.3% were Black/African American. At baseline, pts had an ECOG performance status of 0 (36.6%), 1 (57.7%) or 2 (5.7%); 25.2% of pts had high risk cytogenetics, 15.4% had R-ISS III, and 31.7% had extramedullary disease. Pts had received a median of 5.0 (range, 2−22) prior lines of therapy; 96.7% and 42.3% of pts were triple-class- and penta-drug refractory, respectively.

After a median follow-up of 6.8 months (range, 0.2−16.2), the median duration of elranatamab treatment was 5.3 months (range, 0.03−16.1); 51.2% of pts were still receiving elranatamab at the data cut-off. Most common primary reasons for permanent treatment discontinuation were progressive disease (32.5%) and adverse events (7.3%). The ORR by BICR was 61.0% (95% CI,

51.8−69.6); a clinical benefit was observed across subgroups. Among responders, median time to objective response was 1.2 months (range, 0.9−6.9). The median duration of objective response has not been reached, and the probability of maintaining the response at 6 months was 90.4% (95% CI, 79.8−95.6).

Any grade and grade 3/4 TEAEs were reported in 100% and 74.8% of pts, respectively. Infections were reported in 61.8% (grade 3/4, 31.7%) of pts; most frequently (≥10% of pts) reported were upper respiratory tract infections (14.6% [no grade 3/4]) and pneumonia (10.6% [grade 3/4, 5.7%]). Peripheral neuropathy was reported in 17.1% (grade 3/4, 0.8%) of pts; most common events (≥2% of pts) were peripheral sensory neuropathy (4.9% [no grade 3/4]), paresthesia (4.1% [no grade 3/4]) and gait disturbance (2.4% [no grade 3/4]). Among pts with peripheral neuropathy events (n=21), 47.6% had a medical history of neuropathy. There were 13.8% of pts with TEAEs leading to death, none were assessed as related to elranatamab. Among pts who received the 2-step-up priming regimen (n=119), CRS and ICANS, respectively, were reported in 56.3% and 3.4%; of those pts, 44.8% (n=30/67) and 50.0% (n=2/4) received tocilizumab and/or steroids. All CRS and ICANS were grade 1 or 2. CRS events were confined to the first 2 priming doses (90.6%) and the first 3 doses (98.8%). No pts permanently discontinued treatment due to CRS or ICANS.

These results suggests that subcutaneous 76 mg QW elranatamab is efficacious and has a manageable safety profile in pts with triple-class- and penta-drug refractory MM and no prior BCMA-targeted treatment[31].

During the oral session at the 64th American Society of Hematology (ASH) Annual Meeting and Exposition 2022 a 10.4 month-follow-up data of MagnetisMM-3 were presented. Patients who received elranatamab achieved a high objective response rate of 61%, with 84% probability of maintaining response at nine months. Probability of progression-free survival and overall survival were 63% and 70%, respectively, at nine months. The results also suggest elranatamab has a manageable safety profile and that the two-step-up priming dose regimen (12/32 mg) mitigated the rate and severity of cytokine release syndrome (CRS) (58%, all Grade 1/2) and immune effector cell-associated neurotoxicity syndrome (ICANS, 3%, all Grade 1/2) in cohort A of MagnetisMM-3 (n=119)[32].

3.3. Rationale for the dose of Elranatamab monotherapy

The recommended elranatamab monotherapy dosing regimen is 76 mg QW administered as a SC injection following the 2 step-up priming doses of 12 mg on C1D1 and 32 mg on C1D4.

In Part 1 (dose-escalation) of the FIH Study C1071001, body-weight based doses ranging from 0.1 µg/kg to 50 µg/kg QW IV infusion and 80 µg/kg to 1000 µg/kg QW SC were evaluated in participants with RRMM, the majority of whom were triple-class refractory, to estimate the MTD and select the RP2D. A total of 2 participants in the IV cohorts had TEAEs that were considered DLTs, while no DLTs were observed in the SC cohorts including the highest tested dose of 1000 µg/kg. Therefore, the MTD was not reached in Study C1071001. Based on data available from Study C1071001 during the dose escalation phase, SC dose administration resulted in a lower dose-normalized C_{max} relative to IV administration and no DLTs were observed in the SC cohorts. Therefore, elranatamab will be dosed via SC route in this study.

Population PK analysis based on concentration-time data from all IV and SC cohorts in the dose escalation part of Study C1071001 indicated that body weight is not a clinically relevant covariate on elranatamab exposure. Therefore, a fixed dose will be used in this study.

Encouraging clinical activity was observed in the Phase 1 study (C1071001), particularly at doses ≥215 µg/kg SC with an acceptable safety profile across all dose levels in the dose escalation cohort. The ORR at the highest tested dose of 1000 µg/kg SC was 83.3% (5 of 6 participants).

In terms of safety profile, in the MagnestisMM-1 it was observed how CRS was mostly associated with initial elranatamab exposure. Although it was initially evaluated a single priming dose of 44 mg to mitigate CRS, the selection of two step-up dose levels was supported by an exposure-CRS analysis[35]. The 12/32 mg regimen was predicted to reduce any grade and grade ≥2 CRS, while maintaining the total dose intensity in comparison with a single priming dose of 44 and there was not observed any impact in the efficacy. These results have been recently confirmed in another abstract presented at ASH 2022. Therefore, to maximize the potential for clinical activity while improving the safety margin of elranatamab, a fixed dose equivalent of 1000 µg/kg dose level, 76 mg, was selected as the RP2D with the priming dose regimen to be evaluated in Part 1 of the current study (premedication as well as two step-up priming doses of 12 mg on C1D1 and 32 mg on C1D4 before the full elranatamab dose on C1D8). The dose of elranatamab should be increased to 76 mg on C1D8 as long as the participant meets the criteria listed in Section 5.9. If a participant does not meet these criteria on C1D8, initiation of dosing with 76 mg should be deferred until these criteria are met.

3.4. Rationale for Study Population

In Spain, newly diagnosed MM patients transplant eligible are currently being treated with induction treatment based on PIs, IMiD's with or without anti-CD38 monoclonal antibodies and autologous stem

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cell transplantation followed by maintenance with lenalidomide as continuous therapy until progression, thus becoming lenalidomide-refractory. The newly diagnosed-ones but transplant ineligible patients usually receive treatment with either daratumumab plus VMP or continuous therapy with daratumumab plus Rd, also becoming therefore refractory to lenalidomide and/or daratumumab. In first relapse, combinations based on anti-CD38 monoclonal antibodies, as part of the rescue therapies, are usually used providing the patient had not been previously exposed to CD-38 antibodies in the first line or if the patient is sensitive. Otherwise, there are other combinations like carfilzomib or pomalidomide-based combinations.

Based on the previous background and taking into consideration the most common first lines of therapy used in our country, the majority of MM patients in Spain managed in academic centers would have been, after the first or two first lines of therapy, already exposed to the three main drug classes and the majority of them are lenalidomide-refractory as well as daratumumab-refractory because of their use as continuous therapy.

As mentioned above, in patients with MM who are triple-class refractory, treatment options are very limited and this is the place for the evaluation of the efficacy and safety of elranatamab.

3.5. Rationale for Study Objectives and Study Outcome Measures (Endpoints)

This is a study primary aiming to evaluate the efficacy of elranatamab monotherapy at the dose of 76 mg subcutaneously in participants with RRMM who had received prior treatment with immunomodulatory drugs, protease inhibitors, and anti-CD38 therapy and were refractory to the last line of therapy as evaluated with the rate of complete Response and Undetectable Measurable Residual Disease.

Measurable Residual Disease (MRD) assessment has become essential to enhance the evaluation of treatment efficacy. Detection of MRD has improved with the development of highly sensitive and standardized techniques such as Next Generation Flow or Next Generation Sequencing, complemented by functional imaging techniques. These advances offer a valuable opportunity to further optimize criteria of response to treatment. Currently, there are data supporting that MRD status is a valuable prognostic factor of survival. Thus, a meta-analysis showed a hazard ratio (HR) of 0.44 (95% CI 0.34–0.56, P < .001) for PFS and of 0.47 (95% CI 0.33–0.67, P < .001) for OS in favor of those patients in CR who had undetectable MRD vs. detectable MRD[36]. A pooled analysis of 3 PETHEMA-GEM trials also supports undetectable MRD status as a better prognostic for PFS and OS

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than the CR achievement across the disease spectrum, and regardless of the type of treatment or patient risk group[37].

In this study, we have selected as primary endpoint the rate of undetectable MRD at 6 and 12 months based on the rapid and deep responses so far reported with bispecific monoclonal antibodies to plan an optimization in the duration of treatment based on the achievement of undetectable MRD and sustained over time.

3.6. Added Value for the Study Regarding Previous Trials with Elranatamab

GEM-RANTAB will include patients refractory to less treatment lines than MagnetisMM-3. Our study limits the number of previous lines to 2, whereas previous trials have explored the inclusion of patients triple-refractory, refractory to 3 or more lines of therapy (at least 1 proteasome inhibitor, 1 immunomodulatory drug, and 1 anti-CD38 antibody). The median number of lines in MAGNETISMM-3 was 5, being a population heavily pretreated. The GEM-RANTAB trial will provide information on the efficacy and safety of elranatamab in a population less exposed to previous treatments.

In this study, the value of MRD will be validated as criteria to evaluate efficacy of treatment. In our trial, patients who reach MRD for periods longer than 1 year and seem to have the disease under control will be able to stop treatment. Therefore, a reduction in maximum treatment duration and less late emerging toxicities.

4. PARTICIPANT SELECTION

Participants are eligible to be included in the study only if all of the following criteria apply:

4.1. Inclusion Criteria

a. Male or female, 18 years or older (at the time consent is obtained).
b. Patient who, in the investigator's opinion, is able to comply with the protocol requirements.
c. Prior diagnosis of MM as defined according to IMWG criteria.
d. Patient has given voluntary written informed consent before performance of any study-related procedure not part of normal medical care, with the understanding that consent may be withdrawn by the patient at any time without prejudice to their future medical care.
e. Relapse multiple myeloma patients that have received 1 or 2 prior lines of therapy including at least to one proteasome inhibitor (bortezomib, carfilzomib or ixazomib), one immunomodulatory drug (lenalidomide is mandatory and patients can be also have been exposed to pomalidomide) and at least one anti-CD38 monoclonal antibody (daratumumab or isatuximab).
f. Documented evidence of progressive disease or failure to achieve a response to last line of MM therapy based on investigator's determination of response by IMWG criteria.
g. Patient must have a measurable secretory disease defined as either serum monoclonal protein of ≥ 0,5 g/dl or urine monoclonal (light chain) protein ≥ 200 mg/24 h. For patients in whom disease is only measurable by serum FLC, the involved FLC should be ≥ 10mg/dL (100 mg/L), with an abnormal serum FLC ratio.
h. Patients must have an ECOG performance status of 0 or 1.
i. Left-ventricular ejection fraction (LVEF) ≥40% (MUGA scan or ECHO).
j. Subject must have pretreatment clinical laboratory values meeting the following criteria during the Screening Phase (without transfusion):
    1. hemoglobin ≥8 g/dL
    2. absolute neutrophil count (ANC) ≥ 1.0 x 10^9/L;
    3. platelet count ≥ 25 x10^9/L
    4. aspartate aminotransferase (AST) ≤2.5 x upper limit of normal (ULN);
    5. alanine aminotransferase (ALT) ≤2.5 x ULN;
    6. total bilirubin ≤2 x ULN, except in subjects with congenital hyperbilirubinemia, such as Gilbert syndrome (bilirubin ≤3 x ULN);

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7. estimated creatinine clearance > 30 mL/min/1.73 m2;
8. corrected serum calcium <14 mg/dL (<3.5 mmol/L); or free ionized calcium <6.5 mg/dL (<1.6 mmol/L).

k. Women of childbearing potential must be using a highly effective method of birth control consistent with local regulations regarding the use of birth control methods for subjects participating in clinical studies (see Appendix 13).

l. Women of childbearing potential must have a negative serum pregnancy test at screening within 10-14 days and 24 hours before starting treatment. Females of reproductive potential must agree either to abstain continuously from heterosexual sexual intercourse or to use two methods of reliable birth control simultaneously.

m. Male Participants: Contraceptive use by men or women should be consistent with local regulations regarding the methods of contraception for those participating in clinical studies. Male participants are eligible to participate if they agree to the following during the intervention period and for at least 100 days: refrain from donating sperm PLUS either: be abstinent from heterosexual intercourse as their preferred and usual lifestyle (abstinent on a long term and persistent basis) and agree to remain abstinent OR must agree to use contraception/barrier.

4.2. Exclusion Criteria

a. Subject has a diagnosis of primary amyloidosis, monoclonal gammopathy of undetermined significance (MGUS), smoldering multiple myeloma (SMM), POEMS syndrome (defined by the presence of peripheral neuropathy, organomegaly, endocrinopathy, monoclonal plasma-cells proliferative disorder, and skin changes) or plasma cell leukemia.

b. Prior anti-BCMA treatment.

c. Subject has peripheral neuropathy or neuropathic pain grade 2 or higher, as defined by the National Cancer Institute Terminology Criteria for Adverse Events (NCI CTCAE) Version 5.

d. History of Guillain-Barré syndrome (GBS) or GBS variants, or history of any Grade ≥3 peripheral motor polyneuropathy.

e. Stem cell transplant within 12 weeks prior to enrolment.

f. Active Graft vs. Host Disease (GVHD) (other than Grade 1 skin involvement), or GVHD requiring treatment.

g. Active HBV, HCV, SARS-CoV-2, HIV, or any active, uncontrolled bacterial, fungal, or viral infection. Active infections must be resolved at least 21 days prior to enrollment. Treatment

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with systemic anti-infective agents must have completed at least 28 days prior to enrollment. Prophylactic use of systemic anti-infective agents is permitted. Patients whose HIV or HCV status cannot be determined from their medical history must be screened for HIV and HCV up to 28 days prior to enrollment.

h. Patients who require administration of live attenuated vaccine within 4 weeks of the first dose of study intervention.
i. Prior history of malignancies other than MM, unless the subject has been free of the disease for > 2 years. Exceptions include: basal or squamous cell carcinoma of the skin, carcinoma in situ of the cervix or breast, incidental histological finding of prostate cancer (TNM stage of T1a or T1b).
j. Impaired cardiovascular function and/or clinically significant cardiovascular diseases, defined as any of the following within 6 months: acute myocardial infarction, acute coronary syndromes (e-g- unstable angina, coronary artery bypass graft, coronary angioplasty or stenting); clinically significant cardiac arrhythmias (e.g. uncontrolled atrial fibrillation or uncontrolled paroxysmal supraventricular tachycardia); thromboembolic or cerebrovascular events (e.g. transient ischemic attack, cerebrovascular accident, deep vein thrombosis, or pulmonary embolism); QTcF interval ≥470 msecs.
k. Subject has meningeal involvement of multiple myeloma.
l. Pregnant of lactating females.
m. Known human immunodeficiency virus (HIV) infection, active infectious hepatitis A, B or C or chronic hepatitis B or C.
n. Any serious and/or unstable pre-existing medical, psychiatric disorder or other conditions (including lab abnormalities) not mentioned above that could interfere with participant’s safety, obtaining informed consent or compliance to the study procedures.
o. Subject who received any of the following within the last 14 days of initiation of study treatment: major surgery (except patients recovered from kyphoplasty) or use of any anti-myeloma drug therapy.
p. Investigational drug within prior 30 days or within 5 half-lives of the investigational drug (whichever is longer).

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4.3. Screen Failures

Screen failures are defined as participants who consent to participate in the clinical study but are not subsequently assigned to study intervention. A minimal set of screen failure information is required to ensure transparent reporting of screen failure participants to meet the CONSORT publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, eligibility criteria, and any SAE.

Individuals who do not meet the criteria for participation in this study (screen failure) or individuals who consent but are unable to enroll due to study enrolment hold may be rescreened. Rescreened participants should be assigned a new participant number.

4.4. Lifestyle Considerations

The investigator or his or her designee, in consultation with the participant, will confirm that the participant has selected an appropriate method of contraception for the individual participant and his or her partner(s) from the permitted list of contraception methods and will confirm that the participant has been instructed in its consistent and correct use. At time points indicated in the SoA, (including long term follow-up visit/contacts covering the required contraception period through 6 months post last dose of study intervention the investigator or designee will inform the participant of the need to use highly effective contraception consistently and correctly and document the conversation and the participant’s affirmation in the participant’s chart (participants need to affirm their consistent and correct use of at least 1 of the selected methods of contraception) considering that their risk for pregnancy may have changed since the last visit. In addition, the investigator or designee will instruct the participant to call immediately if the selected contraception method is discontinued or if pregnancy is known or suspected in the participant or partner.

Contraception check is only required in men and WOCBP.

5. Treatment Plan

5.1. Method of Treatment Assignment

Eligible patients shall be enrolled in the study under the supervision of the investigator or designated sub-investigators. If possible, except for the days of hospitalization for monitoring CRS, patients will receive treatment in an outpatient basis.

Patients will be allocated to monotherapy with elranatamab.

5.2. Treatment Administration

For the purposes of this protocol, study intervention refers to elranatamab.

Table 2 Elranatamab pharmaceutical information

	Elranatamab
Dosage form:	Single-use vials
Unit dose strengths:	40 mg/mL solution for injection; 1.9 ml/vial
Route of Administration	SC
Dosing instructions:	Two step-up priming doses of SC elranatamab 12 mg on C1D1 then 32 mg on C1D4 will be administered followed by 76 mg QW beginning C1D8 and continuing through C6, then 76 mg Q2W beginning C7D1 if PR or better for ≥2 months. (Cycles 7-12), and Q4W (once every four weeks, for participants on Q2W schedule) (Cycle 13- onward). Figures 1-3
Manufacturer / Source of Procurement:	Provided centrally by the sponsor

Premedication for CRS prophylaxis is required approximately 60 minutes (±15 minutes) prior to both priming doses (on C1D1 and C1D4) and the first full dose of elranatamab (on C1D8):

- Acetaminophen 650 mg (or paracetamol 500 mg)*
- Diphenhydramine 25 mg (or equivalent)*, oral or IV
- Dexamethasone 20 mg (or equivalent)*, oral or IV

*Different but comparable doses due to local strength variations per prescribing information are permissible.

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Similar premedications for doses at other time points may be given at the discretion of the investigator.

Participants are required to be hospitalized and monitored for CRS/ICANS for at least 2 days for C1D1 and 1 day for C1D4. Hospitalization up to 5 days from C1D1 to C1D5 may be considered. Elranatamab dosing at 32 mg and 76 mg on C1D4 and C1D8, respectively, should proceed only if the participant meets redosing criteria (Section 5.9.1). A minimum of 2 days should be maintained between the 2 step-up doses (C1D1 and C1D4), and a minimum of 3 days between the C1D4 dose and the first full dose (C1D8); a minimum of 6 days should be maintained between doses thereafter.

Two step-up priming doses of SC elranatamab 12 mg on C1D1 then 32 mg on C1D4 will be administered followed by 76 mg QW beginning C1D8 and continuing through C6, then 76 mg Q2W beginning C7D1 if PR or better for ≥2 months. (Cycles 7-12), and Q4W (once every four weeks,

for participants on Q2W schedule) (Cycle 13- onward).

The windows for the different administration schedules are -1/+3 days for the QW schedule, -3 /+7 days for Q2W and -3/+14 days for Q4W.

Qualified and trained investigator site personnel will administer elranatamab to participants by SC injection. Ideally, each injection may be up to 2 mL in volume; however, if the maximum volume allowed per institution's policy is lower than 2 mL, the number of injections may be increased to accommodate this difference in volume and ensure the correct dose is delivered.

The following figures summarized the treatment schedules as well as the premedication strategies.

follow-up

First cycle																												
	1	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	**11**	**12**	**13**	**14**	**15**	**16**	**17**	**18**	**19**	**20**	**21**	**22**	**23**	**24**	**25**	**26**	**27**	**28**
Elranatamab(SC) At 12 mg (1)	■																											
Elranatamab(SC) At 32 mg (1)				■																								
Elranatamab(SC) At 76 mg (1)								■							■							■						
Premedication for CRS prophylaxis (2)	■			■				■							▒							▒						

Figure 1 Summary of Dosing Schedule for first cycle (2 step-up priming regimen).

(1) The dose of elranatamab should be increased from 12 mg to 32 mg and 76 mg on C1D4 and C1D8, respectively, should proceed only if the participant meets redosing criteria.

(2) Premedication consists of 60 minutes (±15 minutes) prior to the first 3 doses of elranatamab

- Acetaminophen 650 mg (or paracetamol 500 mg)
- Diphenhydramine 25 mg (or equivalent), oral or IV
- Dexamethasone 20 mg (or equivalent), oral or IV

Similar premedications for doses at other time points may be given at the discretion of the investigator.

follow-up

Cycles 2 to 6																												
	1	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	**11**	**12**	**13**	**14**	**15**	**16**	**17**	**18**	**19**	**20**	**21**	**22**	**23**	**24**	**25**	**26**	**27**	**28**
Elranatamab(SC) At 76 mg	■							■							■							■						

Figure 2 Summary of Dosing Schedule for cycles 2 to 6.

Cycle 7 onwards																												
	1	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	**11**	**12**	**13**	**14**	**15**	**16**	**17**	**18**	**19**	**20**	**21**	**22**	**23**	**24**	**25**	**26**	**27**	**28**
elranatamab(SC) At 76 mg (1, 2)	■							▒							■							▒						

Figure 3 Summary of Dosing Schedule for cycles 7 onwards.

(1) 76 mg Q2W beginning C7D1 if PR or better for ≥2 months. (Cycles 7-12), and Q4W (once every four weeks, for participants on Q2W schedule) (Cycle 13- onward).

(2) Treatment will be scheduled with a response-adapted duration and patients achieving undetectable measurable residual disease and maintained for 12 months will stop therapy. After stopping therapy, and if the patient is in sustained undetectable measurable residual disease for at least 12 months, it would be possible to re-start treatment with elranatamab in case the measurable residual disease will be detectable or relapse from CR will occur. Patients who will not achieve undetectable measurable residual disease sustained for 12 months will receive continuous treatment until progressive disease. In both situations, the occurrence of unacceptable toxicity might result in treatment discontinuation.

follow-up

Refer to Appendix 11 for details on administration of multiple injections to the abdomen. Study staff should refer to the IP Manuals.

5.3. Packaging and Labeling

All study drugs will be administered as open-label. No blinding methods will be used in this trial.

Experimental drugs will be supplied to pharmacies packaged and labeled according to the standard requirements for clinical trials.

5.4. Preparation, Handling, Storage and Accountability

1. The investigator or designee must confirm appropriate temperature conditions have been maintained during transit for all study interventions received and any discrepancies are reported and resolved before use of the study intervention.
2. Only participants enrolled in the study may receive study intervention and only authorized site staff may supply or administer study intervention. All study interventions must be stored in a secure, environmentally controlled, and monitored (manual or automated recording) area in accordance with the labeled storage conditions with access limited to the investigator and authorized site staff. At a minimum, daily minimum and maximum temperatures for all site storage locations must be documented and available upon request. Data for non-working days must indicate the minimum and maximum temperatures since previously documented for all site storage locations upon return to business.
3. Any excursions from the study intervention label storage conditions should be reported to the sponsor upon discovery along with any actions taken. The site should actively pursue options for returning the study intervention to the storage conditions described in the labeling, as soon as possible. Once an excursion is identified, the study intervention must be quarantined and not used until the sponsor provides permission to use the study intervention. Specific details regarding the definition of an excursion and information the site should report for each excursion will be provided to the site in the IP manual.
4. Any storage conditions stated in the SRSD will be superseded by the storage conditions stated on the label.
5. Study interventions should be stored in their original containers.
6. See the IP manual for storage conditions of the study intervention once prepared for administration.
7. The investigator, or pharmacy designate (where applicable) is responsible for study intervention accountability, reconciliation, and record maintenance (ie, receipt, reconciliation, and final disposition

follow-up

records), such as the investigational product accountability log (IPAL) or sponsor approved equivalent. All study interventions will be accounted for using a study intervention accountability form/record.

8. Further guidance and information for the final disposition of unused study interventions are provided in the IP manual. All destruction must be adequately documented. If destruction is authorized to take place at the investigator site, the investigator must ensure that the materials are destroyed in compliance with applicable environmental regulations, institutional policy, and any special instructions provided by the sponsor.

Upon identification of a product complaint, notify the sponsor within 1 business day of discovery as described in the IP manual.

5.5. Preparation and dispensing

A qualified staff member will dispense via unique container numbers in the vials provided, in quantities appropriate according to the SoA. Refer to the IP manual for dispensation of products.See the IP manual for instructions on how to prepare the study intervention for administration. Study intervention should be prepared and dispensed by an appropriately qualified and experienced member of the study staff (eg, physician, nurse, physician's).

5.6. Disposal

Precaution will be taken to avoid direct contact with the study drug(s). A Material Safety Data Sheet (MSDS) describing occupational hazards and recommended handling precautions will be provided to the investigator. In the case of unintentional occupational exposure, notify the Clinical Coordinators.

Any unused medicinal product should be disposed of at each participating site in accordance with local requirements.

5.7. Study Drugs Compliance

Participants are dosed at the site, so they will receive study intervention directly from the investigator or designee, under medical supervision. The date and time of each dose administered in the clinic will be recorded in the source documents and recorded in the CRF. The dose of study intervention and study participant identification will be confirmed at the time of dosing by a member of the study site staff other than the person administering the study intervention.

follow-up

The site will complete the required dosage Preparation Record located in the IP manual. The use of the Preparation Record is preferred, but it does not preclude the use of an existing appropriate clinical site documentation system. The existing clinical site's documentation system should capture all pertinent/required information on the preparation and administration of the dose. This may be used in place of the Preparation Record after approval from the sponsor and/or designee.

Compliance will be assessed by direct questioning, counting vials, or review during the site visits and documented in the source documents and CRF. Deviation(s) from the prescribed dosage regimen should be recorded in the CRF.

Intervention start and stop dates, including dates for intervention delays and/or dose reductions, will also be recorded in the CRF.

5.8. Concomitant Therapy/Devices

Over the course of this study, additional medications may be required to manage aspects of the disease state of the subjects, including side effects from study treatments or disease progression. Supportive care, including but not limited to antiemetic medications, may be administered at the discretion of the Investigator. Subjects should be discouraged from use of illicit drugs, herbal remedies, self-prescribed drugs, tobacco products, or excessive alcohol at any time during the study.

All medication and procedures, including blood and blood products, used from 28 days prior to screening until 28 days after the last dose of study treatment must be reported on the eCRF.

During this study treatment period:

- Subjects must check with study personnel before initiating the use of prescribed or over-the-counter medications during the study.
- In the event a subject requires a surgical or invasive procedure during the course of the study, the Sponsor, Clinical Coordinators or designee must be contacted to discuss whether or not the subject is eligible to continue.

For information regarding other drugs that may interact with study treatment and affect its metabolism, pharmacokinetics, or excretion, please see the IB and/or SmPC.

5.8.1. Permitted Concomitant Therapy and Premedications

COVID-19 vaccines (approved or given by emergency use authorization) are permitted and should be recorded as concomitant medications (standard AE collection and reporting processes should be followed). The timing of vaccine administration relative to study intervention is at the discretion of the investigator although, if possible, it is best to avoid vaccine administration within 48 hours before or

after the first and second doses of study intervention.

Hormonal contraceptives that meet the requirements of this study are allowed to be used in participants who are WOCBP. For participants who receive oral contraceptives that are metabolized by CYP enzymes, cytokines released during elranatamab treatment may cause drug-drug interaction with hormonal oral contraceptives at both PK and PD levels and may accentuate the side effects associated with oral contraceptives.

The administration of drugs known to cause peripheral neuropathy should be carefully considered, and if possible, avoided by the investigator.

Elranatamab has been demonstrated to transiently increase cytokine levels (eg, IL-6) in vivo in monkeys and humans (also demonstrated via in vitro assays) which is expected with CD3-targeted bsAbs. Cytokines have been shown to result in modest and temporary inhibition of major CYP enzymes (eg, CYP3A4 and CYP2C9). Therefore, treatment with elranatamab has a potential for resulting in modest and temporal increase in the exposure of concomitant medications that are substrates for these enzymes. Caution should be used upon concomitant use of sensitive substrates of CYP enzymes with narrow therapeutic index (eg, CYP3A4: alfentanil, cyclosporine, dihydroergotamine, ergotamine, fentanyl, pimozide, quinidine, sirolimus and tacrolimus; CYP2C9: phenytoin, warfarin) especially during the initial treatment cycle. If the use of warfarin is clinically necessary, caution and additional INR monitoring is recommended during the initial treatment cycle.

See Section 5.2. for details of required premedications for elranatamab.

5.8.2. Prohibited Concomitant Therapies

Administration of live attenuated vaccines is prohibited during study treatment, and for 90 days after the last dose of study treatment. Annual inactivated influenza vaccines are allowed.

No additional anticancer therapy will be permitted while participants are receiving study intervention.

5.8.3. Transfusion Support

Prophylactic use of transfusion support for anemia and/or thrombocytopenia is encouraged, as indicated by the current guidelines of the ASCO and AABB [38].

5.8.4. Hematopoietic Growth Factors

Growth factor support, erythropoietin-stimulating agents, and transfusions are permitted to treat symptoms or signs of neutropenia, anemia, or thrombocytopenia according to local standards of care.

follow-up

Growth factor support should be avoided during the step-up dosing period and through the first treatment dose of elranatamab and also during events of CRS. G-CSF should be considered for Grade 3 neutropenia with infection or fever or any Grade 4 neutropenia.

5.8.5. Supportive Care

Supportive care for disease-related and/or treatment-related symptoms may be administered at the investigator's discretion and according to the specific supportive care product's Prescribing Information.

Allopurinol and/or rasburicase may be administered as needed for tumor lysis prophylaxis or treatment.

Growth factor support, erythropoietin-stimulating agents, and transfusions are permitted to treat symptoms or signs neutropenia, anemia, or thrombocytopenia according to local standards of care.

5.8.6. Corticosteroids

Chronic systemic corticosteroid use for palliative or supportive purposes is not permitted; however, steroid replacement for adrenal insufficiency at doses equivalent to ≤10 mg prednisone daily is acceptable. Acute emergency administration, topical applications, inhaled sprays, eye drops, or local injections of corticosteroids are allowed. Dexamethasone is included in the premedication regimen for elranatamab (Section 5.2).

5.8.7. Surgery

Caution is advised for any surgical procedures during the study. The appropriate interval of time between surgery and study treatment required to minimize the risk of impaired wound healing and bleeding has not been determined. Postoperatively, the decision to reinitiate study treatment should be based on a clinical assessment of satisfactory wound healing and recovery from surgery.

5.8.8. Radiotherapy

Palliative radiotherapy to specific sites of disease in the event of increased localized pain will be permitted if considered medically necessary by the treating physician. Progressive disease should be ruled out. The appropriate interval of time between radiotherapy and study intervention has not been determined.

follow-up

5.8.9. Infection Prophylaxis

The administration of bacterial, fungal and/or viral prophylaxis in accordance with NCCN, ESMO, IMWG and/or local institutional guidelines in participants at increased risk of infection is strongly encouraged[39,40]; according to IMWG guidelines for the management of infections in patients under treatment with BCMA-CD3 monoclonal antibodies[41](Rodriguez P et al. Lancet Oncol 2024), it must be followed:

- Immunoglobulin replacement when IgG levels are lower than 400 mg/dL, at least for the first 6 months.
- Pneumocystis carinii/jirovecii pneumonia prophylaxis with either TMP/SMZ or pentamidine inhalation
- Prophylaxis for herpes zoster reactivation.
- CMV, adenovirus and COVID at screening.

Initiate antiviral prophylaxis to prevent herpes zoster reactivation within 1 week before the start of administration of study treatment and continue for 3 months following study treatment.

Acceptable antiviral therapy includes acyclovir, famcyclovir, or valacyclovir.

Monitor immunoglobulin levels for the occurrence of hypogammaglobulinemia and consider administering intravenous immunoglobulin for IgG level <400 mg/d for at least the first 6 months.

Consider prophylactic administration of antibiotics in patients with cytopenias or at high risk of infection development.

There is not any formal recommendation for antifungal prophylaxis or for monitoring CMV, EBV,… but in case of suspect of infection, the work out recommended should include all tests because some opportunistic infections can occur.

5.9. Dose Modification and Delay

Criteria for dose modification due to toxicity and efficacy are presented in the following sections. The recommended dose modification guidelines for participants who have active confirmed (positive by regulatory authority-approved test) or presumed (test pending/clinical suspicion) SARS-CoV2 infection are commented below.

follow-up

The following is recommended for the administration of study intervention for participants who have active confirmed (positive by regulatory authority-approved test) or presumed (test pending/clinical suspicion) SARS-CoV2 infection:

- For symptomatic participants with active SARS-CoV2 infection, study intervention should be delayed for at least 14 days from the start of symptoms. This delay is intended to allow the resolution of symptoms of SARS-CoV2 infection.
- Prior to restarting treatment, the participant should be afebrile for 72 hours, and SARS-CoV2-related symptoms should have recovered to ≤Grade 1 for a minimum of 72 hours. Notify the study team when treatment is restarted.
- Continue to consider potential DDIs for any concomitant medication administered for treatment of SARS-CoV2 infection.

5.9.1. Re-treatment Criteria

During a cycle: Re-treatment following interruption for treatment-related toxicity and for peripheral neuropathy (any causality) within a cycle should follow the dose modification guidance below.

At the start of a cycle: Re-treatment at the start of any new cycle should not occur until all of the following parameters have been met:

- ANC ≥1 x 10^9/L.
- Platelets count ≥25 x 10^9/L.
- Recovery of treatment-related non-hematologic toxicities to baseline or Grade ≤1 severity.
- For any dosing day (at start of a cycle or during a cycle), no ongoing CRS or ICANS of any grade.
- Recovery of treatment-emergent peripheral neuropathy to Grade <1 severity.

5.9.2. Dose Interruptions/Delays

Doses may be held as needed until toxicity resolution. Appropriate follow-up assessments should be done until adequate recovery occurs as assessed by the investigator.

Missed doses will not be made up and cycles will not be extended to allow for missed doses. A minimum of 2 days should be maintained between the 2 step-up doses (C1D1 and C1D4) and a minimum of 3 days between the C1D4 dose and the first full dose (C1D8); a minimum of 6 days is required between doses thereafter.

follow-up

C1D1 will be based on Day 1 of elranatamab dosing. After C1D4, for each cycle, if elranatamab cannot be administered on the planned day (within the window prespecified in the SoA), it should be skipped until the next planned dose (ie, if Day 15 cannot be administered within ±3 days of planned dose, the dose should be skipped until Day 22).

Participants must receive both elranatamab priming doses before receiving the planned target elranatamab dose for that cohort/arm. If C1D4 cannot be administered within the protocol defined window, elranatamab treatment may restart at the planned C1D4 dose upon meeting the re-treatment criteria. Upon re-treatment, if the C1D4 dose was tolerable, the elranatamab dose should be increased to the planned target dose 4 days later, after which QW dosing with the planned target dose should continue until Q2W is permitted (see Section 5.2).

If elranatamab treatment is interrupted for >2 weeks within the first cycle between the 32 mg priming dose and the first full dose (76 mg), the participant should be retreated with 32 mg elranatamab. If tolerable, the elranatamab dose can be escalated to 76 mg 4 days later.

If re-treatment criteria are not met within 4 weeks of treatment interruption/delay, elranatamab should be permanently discontinued, unless the benefit/risk assessment per the investigator suggests otherwise, in agreement with the sponsor. In the event of a treatment interruption/delay lasting >4 weeks for reasons other than treatment-related toxicity (eg, elective surgery), treatment resumption will be decided in agreement with the sponsor.

5.9.2.1. Dose Modifications for Elranatamab-Related Toxicity

The recommended dose modifications for elranatamab-related toxicities are presented in Table 3. All dose modifications should be based on the worst preceding toxicity and must be recorded on the CRF.

follow-up

Table 3. Dose Modifications for Elranatamab-Related Toxicity and for Peripheral Sensory or Motor Neuropathy

Toxicity	Grade 1[e]	Grade 2	Grade 3	Grade 4
Non-hematologic (excluding peripheral neuropathy – see below)	Continue treatment.	Continue treatment. Alternatively, per investigator discretion, withhold treatment until toxicity is Grade ≤1, then resume treatment [a]	Withhold treatment until toxicity is Grade ≤1 or has returned to baseline, then resume treatment [a,b,g]	Permanently discontinue [a,b,g]
Hematologic [c,d]	Continue treatment.	Continue treatment.	Withhold treatment until toxicity is Grade ≤2 or returns to baseline, then resume treatment [a,d]	Withhold treatment until toxicity is Grade ≤2 or returns to baseline [a,d]
Peripheral sensory or motor neuropathy (all causality) **See Section 7.8 for recommended work-up**	Continue treatment. Continue to monitor the participant for signs of worsening neuropathy.	For new or worsening Grade 2 peripheral neuropathy, withhold treatment until resolution to Grade ≤1, then resume treatment [a] Continue to monitor the participants for signs of worsening neuropathy.f If Grade ≥2 neuropathy reoccurs, permanently discontinue elranatamab.	Permanently discontinue.	Permanently discontinue.

a. Both elranatamab priming doses must be administered before the full elranatamab dose. If C1D4 dosing is interrupted/delayed, elranatamab treatment may restart at the planned 32 mg C1D4 dose upon meeting the re-treatment criteria. If the 32 mg dose is tolerable, elranatamab should be increased to the full 76 mg, 4 days later.

   If after the C1D4 priming dose, a participant experiences a treatment-related AE that leads to dose interruption/delay (ie, dose held on Cycle 1 Day 8), elranatamab treatment may restart at the same dose (32 mg) upon meeting the re-treatment criteria (Section 5.9.1). If the 32 mg dose was tolerable, the elranatamab dose should be increased to 76 mg (depending on cohort) 4 days later.

   If elranatamab treatment is interrupted for >2 weeks within the first cycle between the 32 mg priming dose and the first full dose, the participant should be retreated with 32 mg elranatamab. If tolerable, the elranatamab dose can be escalated to 76 mg (depending on the cohort) 4 days later.

   In case of ongoing CRS or ICANS of any grade on any dosing day, dosing will be held until CRS or ICANS resolution.

   Severity of CRS and ICANS assessed according to ASTCT criteria[42] (Appendix 12).

b. Grade 3 or 4 nausea, vomiting or diarrhea must persist at Grade 3 or 4 despite maximal medical therapy to require dose modification or permanent discontinuation.

c. Excludes lymphopenia which is expected based on the elranatamab mechanism of action.

d. For thrombocytopenia: dosing can continue if platelet re-treatment criteria in Sections 5.9.1, depending on treatment regimen, are met.

e. AEs graded per NCI CTCAE v5.0.

f. Consider additional diagnostic work-up

g. Excludes laboratory abnormalities that are not considered clinically relevant.

follow-up

5.9.2.2. Dose Reductions

Dose reduction of elranatamab is not permitted; dose delay is the primary method for managing elranatamab-related toxicities. Appropriate follow-up assessments should be done until adequate recovery occurs as assessed by the Investigator.

Elranatamab dose levels are outlined in Table 4.

Table 4. Dose Levels for Elranatamab

Dose Level	Remarks
76 mg	Full dose to be administered for elranatamab beginning on C1D8.
32 mg	Dose to be administered on C1D4
12 mg	Dose to be administered on C1D1.

5.10. Treatment of Overdose

For this study, if a participant receives a dose ≥10% higher than the planned dose of elranatamab, it will be considered an overdose.

The sponsor does not recommend specific treatment for an overdose.

In the event of an overdose, the treating physician should:

1. Contact the medical monitor within 24 hours.
2. Closely monitor the participant for any AEs/SAEs and laboratory abnormalities for 90 calendar days after the overdose of elranatamab.
3. Document the quantity of the excess dose as well as the duration of the overdose in the CRF.
4. Overdose is reportable to the Sponsor only when associated with an SAE.
5. Obtain a blood sample for PK analysis within 28 days from the date of the last dose of elranatamab if requested by the medical monitor (determined on a case-by-case basis).

Decisions regarding dose interruptions or modifications will be made by the investigator in consultation with the medical monitor based on the clinical evaluation of the participant.

5.11. Treatment Discontinuation

It may be necessary for a participant to permanently discontinue study intervention. Reasons for permanent discontinuation of study intervention may include the following:

follow-up

- Objective disease progression;
- Global deterioration of health status requiring discontinuation;
- Lack of efficacy (eg, increase of disease burden not qualifying as PD according to IMWG
- criteria);
- Unacceptable toxicity;
- Participant pregnancy or breastfeeding;
- Significant protocol violation;
- Medication error without associated adverse event;
- Lost to follow-up;
- Withdrawal by participant;
- Study terminated by sponsor;
- Investigator decision;
- Completed;
- Death.

Note that discontinuation of study intervention does not represent withdrawal from the study. If study intervention is permanently discontinued, the participant will remain in the study to be evaluated for safety, disease assessments, subsequent anticancer therapies, and survival. Even if the patient discontinued treatment due to withdrawal of consent to any study related activities, it will be recommended to perform a safety evaluation /safety visit. See the SoA for data to be collected at the time of discontinuation of study intervention and follow-up for any further evaluations that need to be completed.

In the event of discontinuation of study intervention, it must be documented on the appropriate CRF/in the medical records whether the participant is discontinuing further receipt of study intervention or also from study procedures, posttreatment study follow-up, and/or future collection of additional information.

5.12. Follow-Up:

At least 28 calendar days, and no more than 35 calendar days after discontinuation of study intervention, participants will return to undergo safety assessments, review of concomitant treatments, vital signs, pregnancy test (if appropriate), contraception check, PROs and assessment for resolution of any treatment-related AEs (see SoA for all activities). Participants continuing to experience AEs at this point following discontinuation of treatment will continue to be followed at least

follow-up

every 4 weeks until resolution or determination, in the clinical judgment of the investigator, that no further improvement is expected. If the unresolved AE is considered by the investigator as possibly related to or associated with ADA formation, the participant will be asked to return for drug concentration and ADA blood sampling at up to 3-month intervals, until the last follow-up of the AE.

New AEs during the 90 day follow up post last administration of study intervention should be collected (also see Section 7.2 for description of AE reporting period).

5.12.1. Long Term Follow-Up:

If the participant has not discontinued from study procedures, posttreatment study follow-up, and/or future collection of additional information, long term follow-up will continue as outlined in the SoA until the end of study (Section 2.1).

5.12.2. Duration of Follow Up

Patients must perform an end of treatment visit (EOT) 30 days after the last dose of study treatment (Section 6.6) After, patients are required to come to a follow-up visit 30 days after EOT (Section 6.6). Afterward a follow-up visit will be performed every three months in order to record patient status, according to routine clinical practice (Section 6.6).

5.13. Discontinuation from Study

A participant may withdraw from the study at any time at their own request. Reasons for discontinuation from the study may include:

- Withdrawal by participant;
- Lost to follow-up;
- Death;
- Study terminated by sponsor;
- Study Completed.

If a participant withdraws from the study, they may request destruction of any remaining samples taken and not tested, and the investigator must document any such requests in the site study records and notify the sponsor accordingly.

If the participant withdraws from the study and also withdraws consent for disclosure of future information, no further evaluations should be performed, and no additional data should be collected. The sponsor may retain and continue to use any data collected before such withdrawal of consent.

follow-up

5.14. Withdrawal of Consent

Participants who request to discontinue receipt of study intervention will remain in the study and must continue to be followed for protocol-specified follow-up procedures. The only exception to this is when a participant specifically withdraws consent for any further contact with him or her or persons previously authorized by the participant to provide this information. Participants should notify the investigator in writing of the decision to withdraw consent from future follow-up, whenever possible. The withdrawal of consent should be explained in detail in the medical records by the investigator, as to whether the withdrawal is only from further receipt of study intervention or also from study procedures and/or posttreatment study follow-up and entered on the appropriate CRF page. In the event that vital status (whether the participant is alive or dead) is being measured, publicly available information should be used to determine vital status only as appropriately directed in accordance with local law.

5.15. Lost to Follow Up

A participant will be considered lost to follow-up if he or she repeatedly fails to return for scheduled visits and is unable to be contacted by the study site.

The following actions must be taken if a participant fails to return to the clinic for a required study visit:

- The site must attempt to contact the participant and reschedule the missed visit as soon as possible and counsel the participant on the importance of maintaining the assigned visit schedule and ascertain whether or not the participant wishes to and/or should continue in the study.
- Before a participant is deemed lost to follow up, the investigator or designee must make every effort to regain contact with the participant (where possible, via e-mail or telephone call). These contact attempts should be documented in the participant’s medical record.
- Should the participant continue to be unreachable, he/she will be considered to have withdrawn from the study.

5.16. Participant Replacement Criteria

Subjects who do not receive at least one dose of study treatment or who are screening failure will be replaced. Subjects who withdraw will not be replaced.

5.17. Interventions after the End of Study

There is no planned intervention after the End of Study.

follow-up

The investigator is responsible for ensuring that consideration has been given to the post-study care of the participant's medical condition.

Refer to the Schedule of Assessments (SoA, Section 6) for follow-up assessment of participants who are to be followed for disease progression and survival after they permanently discontinue from study treatment.

follow-up

6. Schedule of Assessments

A signed, written informed consent form must be obtained from the participant prior to any study-specific procedures or assessments being performed.

Assessments must be performed prior to administration of any study agent.

Whenever vital signs, 12-lead electrocardiograms (ECGs), and blood draws are scheduled for the same nominal time, the assessments must occur in the following order: 12-lead ECG, vital signs, blood draws. The timing of the assessments must allow the blood draw to occur at the exact nominal time.

- Study procedures and their timing are summarized in the SoA.
- Protocol waivers or exemptions are not allowed.
- Demographic and baseline assessments will include year of birth, sex,
- Medical/medication/family history will be assessed as related to the inclusion/exclusion criteria listed in Section 4.
- Immediate safety concerns must be discussed with the clinical coordinator or sponsor immediately upon occurrence or awareness to determine if the participant should continue or discontinue study treatment.
- Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct.
- All screening evaluations must be completed and reviewed to confirm that potential participants meet all eligibility criteria. The investigator will maintain a screening log to record details of all participants screened and to confirm eligibility or record reasons for screening failure, as applicable.
- Procedures conducted as part of the participant's routine clinical management (e.g., blood count) and obtained before signing of ICF may be utilized for screening or baseline purposes provided the procedure met the protocol-specified criteria and was performed within the time frame defined in the SoA (Table 5). If the time of the imaging test exceeds 28 days prior to the initiation of treatment, it must be discussed with the clinical coordinator so as to be accepted.
- Survival follow-up can be conducted by chart review, phone call, or any other form of communication as laid out in the SoA (Table 5).
- Visit windows:
    - Baseline disease assessments must be completed within 28 days prior to dosing start unless otherwise specified. Refer to the SoA

follow-up

- Screening assessments performed within the permitted time may not be repeated on C1D1 unless otherwise specified.
- Safety labs completed within 72 hours of the first dose do not need to be repeated on C1D1.
- Pregnancy testing must be completed within 10-14 days prior to initiation of treatment and the second test in the 24 hours prior to first dose.
- Imaging must be completed within 28 days prior to the first dose.
- On-study Q4W assessments have a ± 5 day window.
- Overall Survival Follow-up Visits have a ± 14 day window.

An overview of study assessments/procedures is provided below:

Table 5 Study schedule

Cycle	Screening	C1-C6					C7+				EOT	Follow-up
Day	-28 to -1	D1	D4	D8	D15	D22	D1	D8	D15	D22	*	
Informed consent	X											
Medical history	X											
Physical exam (1)	X	X					X				X	
Vital signs (2)	X	X	X (15)	X (15)			X				X	
ECOG performance status	X	X					X					
Electrocardiogram (3)	X										X	
Echocardiogram / MUGA (4)	X											
Premedication for elranatamab (5)		C1	C1	C1								
PET-TC (6)	X											
Bone disease evaluation: WBLDCT or PET-CT (7)	X	X (7)					X (7)				X	
Evaluation of plasmacytomas (8)	X										X	
Blood sample: complete blood count	X	X	C1	C1-2	C1-3	C1-2	X				X	
Blood sample: biochemistry	X	X			X		X				X	
Serology to rule out HBV, HCV and HIV	X											
Urinalysis	X										X	
Albumin 24-hour urine determination	X	X					X				X	
Beta-2-microglobulin	X	X					X					

Cycle	Screening	C1-C6					C7+				EOT	Follow-up
Day	-28 to -1	D1	D4	D8	D15	D22	D1	D8	D15	D22	*	
SPEP/IF (9)	X	X					X				X	
Serum Immunoglobulins (IgG, IgA, IgM)	X	X					X					
Pregnancy test (10)	X	X					X				X	
UPEP/IF (11)	X	X					X				X	
sFLC (12)	X	X					X				X	
CRP, PT/INR, aPTT, fibrinogen, d-dimer	X	X					X				X	
LDH	X											
Bone marrow aspiration (13)	X	X					X				X	
Concomitant medication	X											
Adverse events (14)	X	X	X	X	X	X	X	X	X	X	X	
Survival, PFS and Safety Follow-up												X

(1) A complete physical exam will be carried out during the screening visit and thereafter targeting symptoms in subsequent study visits.
(2) Vital sign measurements must include systolic and diastolic blood pressure, temperature, pulse rate, respiratory rate and oxygen saturation. Vital signs must be measured after resting for at least 5 minutes. Vital signs must be measured more frequently if warranted by the clinical condition of the participant. Vital signs are to be monitored at least every 4 hours (± 15 min) during first 48 hours after the first dose of study intervention (C1D1) and 24 hours after second dose of study intervention (C1D4)

(3) This test (ECG) will be repeated throughout the study as necessary. Whenever vital signs, 12-lead electrocardiograms (ECGs), and blood draws are scheduled for the same nominal time, the assessments must occur in the following order: 12-lead ECG, vital signs, and blood draws. The timing of the assessments must allow the blood draw to occur at the exact nominal time

(4) This test (ECHO) will be repeated when clinically indicated based on patient condition. A MUGA scan is also valid.

(5) Required for priming doses and first full dose of elranatamab.

(6) Patients suspected of having extramedullary disease will undergo PET/TC, contrasted whole body CT scan or whole-body MRI as per investigation discretion and centre availability. The imaging evaluation should be performed every 12 weeks, for disease evaluation or before if it is clinically indicated. The same imaging technique should be use throughout disease course for disease evaluation. Once the patient is in complete metabolic response, it is not necessary to repeat it every 12 weeks but once per year unless it is indicated by clinical symptomatology at investigator criteria.

(7) At screening, a series of bone x-rays or a low-dose CT or PET-CT scan is mandatory. This will be repeated at the investigator's discretion.

(8) Patients suspected of having extramedullary disease will undergo PET/TC, contrasted whole body CT scan or whole body MRI as per investigation discretion and center availability. The imaging evaluation should be performed every 12 weeks, for disease evaluation or before if it is clinically indicated. The same imaging technique should be use throughout disease course for disease evaluation.

(9) After the end of treatment, SPEP/IF will be evaluated every 3 months according to standard routine clinical practice. For patients who discontinue study intervention without PD: perform at EOT, then at least Q4W (±1 wk) until PD according to routine clinical practice.

(10) Females of childbearing potential must have two negative pregnancy tests (sensitivity of at least 25 mIU/mL) prior to starting elranatamab. The first pregnancy test must be performed within 10 to 14 days prior to the start of elranatamab and the second pregnancy test must be performed within 24 hours prior to the start of elranatamab. The subject may not receive elranatamab until the study doctor has verified that the results of these pregnancy tests are negative. The duration of the contraception required for WOCBP after the last dose of the elranatamab study intervention is 5 months.
Females of childbearing potential with regular or no menstrual cycles must agree to have pregnancy tests weekly for the first 28 days of study participation and then every 28 days while taking elranatamab, at study discontinuation, and at Day 30 following the last dose of elranatamab.
Females of childbearing potential with irregular menstrual cycles must agree to have pregnancy tests weekly for the first 28 days of study participation and then every 14 days while taking elranatamab, at study discontinuation, and at Days 14 and 30 following the last dose of elranatamab.

(11) After the end of treatment, UPEP/IF will be evaluated every 3 months according to standard routine clinical practice. For patients who discontinue study intervention without PD: perform at EOT, then at least Q4W (±1 wk) until PD according to routine clinical practice.

(12) Determination of serum free light chains (sFLC) will be carried out at the screening visit and the time of documenting CR by negative IF. For those patients for which this is the detection technique for measurable disease (oligosecretory or nonsecretory MM), determination of sFLC will be carried out on day 1 of each new treatment cycle (± 4 days) and at the end of treatment.

(13) BM tests will include morphological analyses, cytogenetic tests, molecular biology, and flow cytometry. BM aspiration will be done at screening visit, after 6 months, 12 months, at suspected CR and yearly (±1 month) after achieving CR.

(14) This applies to all AEs, including second primary malignancies, whether or not they are related to the study drugs, and that occur up to 30 days after that last dose of study drug is received, and those that occur at any time that are suspected to be related to the study drug.

(15) Only during cycle 1

(C) Overall Survival Follow-up visits will be performed every three months (± 14 days) to record patient status.
(*) Within 30 Days of Last Dose.

6.1. Screening / baseline visit

The patient must sign and date the informed consent document before any study procedure can be carried out. However, all procedures that are part of clinical practice related to the diagnosis of multiple myeloma and that are carried out before the informed consent document is signed will be valid as long as these are done within the 28 days prior to start of study treatment.

During the screening visit the patient must sign the informed consent form in order to participate in the study, and before any type of analysis or other evaluations can be performed. All of the screening visit tests will be carried out before the patient receives any study medication.

- Informed consent signature.
- Medical history, including all the demographic data (year of birth, sex, and race), a summary of the pathology, concomitant disorders and medications, concomitant treatments and baseline diseases.
- Anamnesis and complete physical exam (including weight).
- Vital signs.
- Electrocardiogram
- Echocardiogram. A MUGA scan is also valid.
- Pregnancy test in female of childbearing potential, as well as gathering information on the use of reliable birth control methods.
- Evaluation of plasmacytomas
- PET/CT to evaluate the presence of active disease with or without osteolytic bone lesions. If PET/Ct would not be available, low dose CT will be the next option.
- In patients with known or suspected extra or paramedullary disease a whole body PET-CT scan, whole body MRI or contrast-enhanced whole body CT scan are required at screening.
- ECOG performance status
- Collection of blood samples for analysis: total blood count, biochemistry tests
- Serology to rule out HBV, HCV and HIV.
- SPEP/IF. Serum electrophoresis and immunofixation to quantify the amount and type of M-component.
- Dosage of serum immunoglobulins: IgG, IgA, IgM.
- Albumin in 24-hour urine determination
- Urinalysis

- UPEP/IF. Urine electrophoresis in 24-h collected urine sample to detect, quantify and identify the presence and type of Bence Jones proteinuria
- Serum free light chains (sFLC).
- Collection of blood samples to determine C-reactive protein (CRP), LDH, B2 microglobulin, PT/INR, aPTT, fibrinogen, d-dimer
- Bone marrow (BM) aspiration. BM tests will include morphological analyses, cytogenetic and molecular biology tests as well as flow cytometry. All tests will be carried out centrally at reference laboratories, except morphological analyses.
- Documentation of concomitant medications and adjunctive treatments.
- Documentation of AEs as per the National Cancer Institute's Common Terminology Criteria for Adverse Events (CTCAE) (APPENDIX 3) version 5.0.

6.2. Cycles 1-6

During the treatment period, the following analytical samples will be taken and the following tests will be performed on day 1:

- Physical exam.
- Vital signs
- ECOG performance status.
- Collection of blood samples to determine C-reactive protein (*) and B2 microglobulin, PT/INR, aPTT, fibrinogen, d-dimer.
- Albumin in 24-hour urine determination.
- Pregnancy test in female of childbearing potential, as well as gathering information on the use of reliable birth control methods.
- Blood samples will be collected for the following laboratory tests: complete blood count and biochemistry. (*) Blood sample for blood count should be performed on day 1 of every cycle, day 4 for cycle 1, day 8 for cycles 1-2, day 15 for cycles 1-3, and day 22 for cycle 1-2
- SPEP with immunoglobulins quantification is required on the day 1 of each cycle.
- Determination of serum free light chains (sFLC) will be carried out at the screening visit and the time of documenting CR by negative IF. For those patients for which this is the detection technique for measurable disease (oligosecretory or nonsecretory MM), determination of sFLC will be carried out on day 1 of each new treatment cycle (± 4 days) and at the end of treatment.

- Dosage of serum immunoglobulins: IgG, IgA, IgM.
- BM aspiration should be carried out at the time of suspected CR, at months 6 and 12 (see APPENDIX 2 and 11). All tests will be carried out centrally at reference laboratories, except morphological analyses.
- UPEP. A 24-hour urine sample will be collected to detect the presence of proteinuria, evaluate the monoclonal protein and carry out immunofixation tests on day 1 of each new treatment cycle. Urine immunofixation will be required for those patients with absence of bence jones proteinuria in the UPEP.
- Those patients who show a positive result during screening in the PET-CT will undergo another PET/CT at the time of reporting CR in order to make a proper response assessment. If the PET/CT remains positive, it should be repeated every 3 months. Once PET/CT is negative, it should be also repeated annually to evaluate the category of sustained MRD negativity. However, an imaging test will be carried out, at the investigator's discretion, any time that progression to symptomatic disease is suspected.
- Documentation of AEs as per the National Cancer Institute's Common Terminology Criteria for Adverse Events (CTCAE) (APPENDIX 3) version 5.0. (*)

All of these tests are mandatory on day 1 of each new cycle.

(*) Some tests should be repeated on day fifteen of every cycle. A margin of +/- 3 days is allowed to carry out the required tests.

6.3. Cycles 7+

During the treatment period (Cycle 7+), the following analytical samples will be taken and the following tests will be performed on day 1:

- Physical exam.
- Vital signs
- ECOG performance status.
- Blood sample: complete blood count and biochemistry and B2 microglobulin, C-reactive protein, PT/INR, aPTT, fibrinogen, d-dimer.
- Albumin in 24-hour urine determination
- SPEP with immunoglobulins quantification is required on the day 1 of each cycle.

- Determination of serum free light chains (sFLC) will be carried out at the screening visit and the time of documenting CR by negative IF. For those patients for which this is the detection technique for measurable disease (oligosecretory or nonsecretory MM), determination of sFLC will be carried out on day 1 of each new treatment cycle (± 4 days) and at the end of treatment.
- Dosage of serum immunoglobulins: IgG, IgA, IgM.
- Pregnancy test in female of childbearing potential, as well as gathering information on the use of reliable birth control methods.
- UPEP. A 24-hour urine sample will be collected to detect the presence of proteinuria, evaluate the monoclonal protein and carry out immunofixation tests on day 1 of each new treatment cycle. Urine immunofixation will be required for those patients with absence of bence jones proteinuria in the UPEP.
- Those patients who show a positive result during screening in the PET-CT will undergo another PET/CT at the time of reporting CR in order to make a proper response assessment. If the PET/CT remains positive, it should be repeated every 3 months. Once PET/CT is negative, it should be also repeated annually to evaluate the category of sustained MRD negativity. However, an imaging test will be carried out, at the investigator's discretion, any time that progression to symptomatic disease is suspected.
- BM aspiration should be carried out at the time of suspected CR, at months 18, 24, yearly thereafter, and at the time of disease progression. Optional: at EOT if PD MRd-ve rate will be reported as the % of patients in MRd-ve. (see APPENDIX 2 and 10). All tests (including mass spectrometry) will be carried out centrally at reference laboratories, except morphological analyses.
- Documentation of AEs as per the National Cancer Institute's Common Terminology Criteria for Adverse Events (CTCAE) (APPENDIX 3) version 5.0.

All of these tests are mandatory on day 1 of each new cycle. Some tests may be repeated more often if clinically indicated. A margin of +/- 3 days is allowed to carry out the required tests.

6.4. End of treatment visit (EOT) or treatment discontinuation visit

Patients who discontinue the treatment for any reason should perform an end of study visit **30 days (+/- 5 days) after the last dose of treatment**. A disease workup is required which will include:

- Vital signs.
- Physical exam.
- Electrocardiogram.
- Bone disease evaluation: WBLDCT or PET-CT.
- Evaluation of plasmacytomas.
- Collection of blood samples for the following laboratory tests: complete blood count and biochemistry and C-reactive protein, PT/INR, aPTT, fibrinogen, d-dimer.
- Pregnancy test.
- Urinalysis.
- Albumin in 24-hour urine determination.
- SPEP/IF. Collection of blood samples to evaluate monoclonal protein, quantify immunoglobulins and perform immunofixation tests if the monoclonal protein has disappeared during electrophoresis.
- UPEP/IF. Collection of 24-hr urine sample to determine the presence of proteinuria, evaluate the monoclonal protein, and for immunofixation tests.
- BM aspiration to carry out morphological analysis, flow cytometry, and molecular biology tests if reason of discontinuation was disease progression and at investigator criteria.
- Determination of serum free light chain (sFLC) concentration.
- Documentation of AEs in accordance with CTCAE criteria v5.0.

A window of +/- 7 days is permitted to complete the required tests.

6.5. Follow-up visits (Survival) every 3 months (+/- 14 days)

A follow-up visit will be performed every three months in order to record patient status, according to routine clinical practice.

The visit will include:

- Date of progression (PFS).
- Date salvage therapy is initiated.
- Survival.
- If hematological or non-hematological second primary malignancies are detected during the follow-up period should also be documented.

6.6. End of Study Definition

For the study as a whole, the end of study is defined as the last patient of the last visit (LPLV), expected for 3Q 2029.

6.7. Assessments

6.7.1. Medical history and physical examination

A medical history will be obtained during the screening visit. The Investigator or designee will collect relevant medical history before the start of study treatment. Relevant medical history will include information on the subject's concurrent medical conditions. The medical history will include all the demographic data and a summary of the pathology.

The baseline physical examination will be a complete physical examination (including weight...).

During all visits, a physical examination is mandatory aimed according to the symptoms indicated by the patient.

6.7.2. Vital Signs

Vital sign measurements must include systolic and diastolic blood pressure, temperature, pulse rate, respiratory rate and oxygen saturation. Vital signs must be measured after resting for at least 5 minutes. Vital signs must be measured more frequently if warranted by the clinical condition of the participant. Vital signs are to be monitored at least every 4 hours (± 15 min) during first 48 hours after the first dose of study intervention (C1D1) and 24 hours after the second dose of study intervention (C1D4).

6.7.3. Performance Status

ECOG Performance Status will be determined using the score present in the APPENDIX 4.

6.7.4. Laboratory assessments: hemogram, biochemistry and pregnancy test

The following blood samples for laboratory assessments will be performed during the treatment when required, on the screening visit, and during the treatment period, on day 1 of each cycle (± 3 days). These assessments will be repeated at the discretion of the investigator if there is a clinical reason.

The following lab parameters will be assessed:

- **Hemogram**: Complete blood count, hemoglobin, white blood cell count with differential count and platelet count.
- **Biochemistry**: Triglycerides, ferritin, troponin, NT-proBNP, urea, creatinine, uric acid, bilirubin, alkaline phosphatase, LDH, AST, ALT, sodium, chloride, calcium, potassium and, glucose, magnesium, GGT, CRP and CPK
- Serology to rule out HBV, HCV and HIV. (Screening visit)
- **Medically supervised pregnancy test**, in females with childbearing potential with a minimum sensitivity of 25 IU/ml. A pregnancy test (serum or urine) will be carried out only in female of childbearing potential prior to starting elranatamab. The test will not be necessary if menopause is confirmed, defined as the absence of menstruation for more than 2 years, or surgically-induced menopause.

  The first pregnancy test must be performed within 10 to 14 days prior to the start of elranatamab and the second pregnancy test must be performed within 24 hours prior to the start of elranatamab. The subject may not receive elranatamab until the study doctor has verified that the results of these pregnancy tests are negative.

  Females of childbearing potential with regular or no menstrual cycles must agree to have pregnancy tests weekly for the first 28 days of study participation and then every 28 days while taking elranatamab, at study discontinuation, and at Day 30 following the last dose of elranatamab.

  Females of childbearing potential with irregular menstrual cycles must agree to have pregnancy tests weekly for the first 28 days of study participation and then every 14 days while taking elranatamab, at study discontinuation, and at Days 14 and 30 following the last dose of elranatamab.

6.7.5. Electrocardiogram

During the screening visit, an electrocardiogram must be performed, and will be repeated when clinically indicated.

6.7.6. Echocardiogram

An echocardiogram will be performed during the screening visit, and when clinically indicated based on patient condition. A MUGA scan is also valid.

6.7.7. Serum protein electrophoresis (SPEP)/Immunofixation (IF)

Blood samples will be collected for immunoglobulin quantification as well as monoclonal component quantification and identification. Immunofixation will be necessary only if electrophoresis is negative.

After the end of treatment, SPEP and IF, if applicable, will be repeated every 3 months according to standard routine clinical practice.

6.7.8. Urine protein electrophoresis (UPEP)/Immunofixation (IF)

24-hour urine samples will be collected for Bence Jones protein identification and quantification during the screening period and during treatment.

After the end of treatment, UPEP/IF will be evaluated every 3 months according to standard routine clinical practice.

6.7.9. Serum free light chains

Determination of serum free light chains (sFLC) will be carried out at the screening visit and the time of documenting CR by negative IF.

For those patients for whom this is the detection technique for measurable disease (oligosecretory/non-secretory MM or pure Bence Jones MM) sFLC assessments will be performed during treatment period (day 1 of each new treatment cycle ± 4 days) and at the end of treatment. In the rest of patients with measurable disease by conventional techniques, it will only be mandatory at the time of documenting CR by negative IF in order to evaluate if the response qualifies for stringent CR.

6.7.10. BM Aspirate

A BM aspirate will be performed to all patients during the screening visit, at months 6, 12, at suspected CR and yearly (±1 month) after achieving CR, and at the time of disease progression. In case of achievement of CR close (equal or less than 2 months) to anyone of the scheduled BM aspirates, it is not necessary to repeat it.

BM assessments will include: morphology, flow cytometry, cytogenetic and molecular biology. All assessments will be performed in a centralized manner in reference laboratories.

6.7.11. Bone disease evaluation: low-dose CT or PET/CT

It is highly recommended to use more sensitive techniques, like low-dose CT or PET-CT for the evaluation of bone disease. PET-CT would be the first recommendation.

If PET-CT is the imaging test available, it should be repeated at the moment of CR to evaluate the MRD by imaging. This is applicable to patients with positive uptake at screening.

Sometimes patients will present disease progression which manifests through pain symptoms due to bone alterations. Thus, progression of disease may be documented in these cases through bone surveys or other radiographic assessments, depending on the symptoms experienced by the patient. If the progression of disease diagnosis is obvious in the radiographic assessments, it is not necessary to repeat radiographic assessments for confirmation. In those cases, in which changes are more subtle, it is possible that it will be necessary to repeat radiographic assessments during 1-3 weeks.

During the screening visit, it is important to document the myeloma disease locations, especially in extramedullary areas. It is possible that this documentation requires the performance of clinical examinations, computerized tomography or magnetic resonance imaging assessments. In these cases, in order to document the response, it is necessary to repeat the same test performed for its documentation and assessment.

6.7.12. Beta-2 microglobulin and C reactive Protein

Blood samples for beta-2 microglobulin and reactive C protein must be performed during the screening visit, at day 1 of every treatment cycle and at any other time during the study, at the discretion of the investigator.

6.7.13. PET/CT

During the screening visit, PET/CT must be carried out. Those patients who show a positive result during screening in the PET-CT will undergo another PET/CT at the time of reporting CR in order to make a proper response assessment. If the PET/CT remains positive, it should be repeated every 3 months. Once PET/CT is negative, it should be also repeated annually to evaluate the category of sustained MRD negativity. However, an imaging test will be carried out, at the investigator’s discretion, any time that progression to symptomatic disease is suspected.

6.7.14. Safety Assessments

Planned time points for all safety assessments are provided in the SoA.
Safety assessments include the following:

- Complete physical examinations, including weight (kg)
- Vital signs
- ECOG performance status

- Clinical chemistry, hematology, and other laboratory tests
- Serum beta-human chorionic gonadotropin (β -HCG) pregnancy test for female participants of childbearing potential only
- 12-lead ECG and echocardiogram at Screening.

Pharmacovigilance during the trial will be carried out pursuant to regulations established in Royal Decree 1090/2015 on Clinical Trials on recording, evaluating and reporting of adverse events. The sponsor, through the principal investigator, will evaluate Serious Adverse Events (SAE) using the product safety data sheet or the Investigators Brochure. With a prior assessment of Clinical Coordinators, the Sponsor will report, via the CRO, those SAEs that meet the expedited reporting criteria (serious, unexpected and related to the treatments) to the health authorities. The sponsor will also be responsible for notifying the competent authorities [AEMPS (Spanish Agency of Medicines and Health Products)], competent authorities of the autonomous communities) and the principal investigators (see APPENDIX 8).

Refer to the SoA for the timing and frequency Clinical laboratory tests to be performed. Details for the preparation and shipment of samples for central laboratory assessments will be provided in the Study Reference Manual (SRM)/Clinical Lab Manual. The investigator must review the laboratory report, document this review, and record any clinically relevant changes occurring during the study in the AE section of the eCRF. The laboratory reports must be filed with the source documents. Clinically significant abnormal laboratory findings are those which are not associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.

6.7.15. Pregnancy Testing (FCBP Only)

All females of childbearing potential, including those who commit to total abstinence, must undergo medically-supervised pregnancy tests with a minimum sensitivity of 25 mIU/mL, as detailed below in Table 5 (Study Schedule). The duration of the contraception required for FCBP participants after the last dose of the elranatamab study intervention is 5 months.

Before beginning treatment with the study drug

Female patients:

- All FCBP must have two negative pregnancy tests (with a sensitivity of at least 25 mIU/mL) before starting the study treatment. The first pregnancy test should be performed 10-14 days

prior to initiation of treatment, and the second test in the 24 hours prior to initiation of treatment. The patient cannot receive treatment with the study drug until the investigator has verified that the results of these pregnancy tests are negative.

During participation in the study and for 30 days following the end of treatment with the study drug

Female patients:

- All FCBP with regular menstrual cycles or who have no menstrual cycles agree to undergo pregnancy tests weekly during the first 28 days of their participation in the study, every 28 days thereafter throughout the study, after suspension of the study treatment, and at 30 days after treatment has finished.
- If the patient's menstrual cycles are irregular, pregnancy tests should be carried out once a week during the first 28 days, then every 14 days over the course of study, at the time treatment with the study drug has stopped, and at 14 and 30 days thereafter.

7. Adverse Event Reporting

7.1. Definition of Adverse Events

An adverse event (AE) is any untoward medical occurrence in a clinical study participant, temporally associated with the use of a study treatment, whether or not considered related to the study treatment.

Note: An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of a study treatment.

7.1.1. Events Meeting the AE Definition

- Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments (e.g., ECG, radiological scans, vital signs measurements), including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator (i.e., not related to progression of underlying disease).
- Exacerbation of a chronic or intermittent pre-existing condition including either an increase in frequency and/or intensity of the condition.
- New conditions detected or diagnosed after study treatment administration even though it may have been present before the start of the study.
- Signs, symptoms, or the clinical sequelae of a suspected drug-drug interaction.
- Signs, symptoms, or the clinical sequelae of a suspected overdose of either study treatment or a concomitant medication. Overdose per se will not be reported as an AE/SAE unless it is an intentional overdose taken with possible suicidal/self-harming intent. Such overdoses must be reported regardless of sequelae.

"Lack of efficacy" or "failure of expected pharmacological action" per se will not be reported as an AE or SAE. Such instances will be captured in the efficacy assessments.

7.1.2. Events NOT Meeting the AE Definition

- Any clinically significant abnormal laboratory findings or other abnormal safety assessments which are associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.
- The disease/disorder being studied or expected progression, signs, or symptoms of the disease/disorder being studied, unless more severe than expected for the participant's condition.
- Medical or surgical procedure (e.g., endoscopy, appendectomy): the condition that leads to the procedure is the AE.
- Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital).
- Anticipated day-to-day fluctuations of pre-existing disease(s) or condition(s) present or detected at the start of the study that do not worsen.

7.2. Serious Adverse Events (SAEs)

If an event is not an AE per definition above, then it cannot be an SAE even if serious conditions are met (e.g., hospitalization for signs/symptoms of the disease under study, death due to progression of disease).

Any untoward medical occurrence that, at any dose;

- Results in death;
- Is life threatening (i.e., an event in which the patient was at risk of death at the time of the event; it does not refer to an event that hypothetically might have caused death if it were more severe);
- Requires inpatient hospitalization* or prolongation of existing hospitalization;
- Results in persistent or significant disability/incapacity;
- Is a congenital anomaly/birth defect;
- Is an important medical event**;

- Is associated with liver injury and impaired liver function defined as:
  - ALT ≥3 x ULN and total bilirubin[a] ≥2 x ULN (>35% direct), or
  - ALT ≥3 x ULN and INR[b] >1.5.

  [a] Serum bilirubin fractionation should be performed if testing is available; if unavailable, measure urinary bilirubin via dipstick. If fractionation is unavailable and ALT ≥3 x ULN and total bilirubin ≥2 x ULN, then the event is still to be reported as an SAE.

  [b] INR testing not required per protocol and the threshold value does not apply to participants receiving anticoagulants. If INR measurement is obtained, the value is to be recorded on the SAE form.
- Any suspected transmission of any infectious agent via a product (medically significant)**.

*Exception: Preplanned (at time of informed consent) hospitalization for elective procedures, for protocol compliance or social reasons, or for observation will not be considered criteria for an SAE. The reason for the planned hospitalization should be documented. Complications experienced during these hospitalizations must be reported as SAEs if hospitalization is prolonged due to AE, or if the complication meets other serious criteria).

**Medical and scientific judgment should be exercised in determining whether situations or events should be considered serious adverse events: an important medical event may not be immediately life-threatening or result in death or require hospitalization but may jeopardize the patient or require intervention to prevent one of the above outcomes. Examples of such events are allergic bronchospasm, blood dyscrasias, or convulsions that may require intensive treatment in an emergency room or at home but do not result in hospitalization, development of drug dependency or drug abuse, and transmission of disease associated with the administration of the study drug.

7.3. Time Period and Frequency for Collecting AE and SAE Information

The time period for actively eliciting and collecting AEs and SAEs ('active collection period') for each participant begins from the time the participant provides informed consent, which is obtained before the participant's participation in the study (ie, before undergoing any study-related procedure and/or receiving study intervention), through and including a minimum of 90 calendar days, except as

indicated below, after the last administration of the study intervention. NOTE, if a participant begins a new anticancer therapy, the recording period for non serious AEs ends at the time the new treatment is started; however, SAEs must continue to be recorded on the CRF during the above-indicated active collection period.

Follow-up by the investigator continues throughout and after the active collection period and until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator.

For participants who are screen failures, the active collection period ends when screen failure status is determined.

If the participant withdraws from the study and also withdraws consent for the collection of future information, the active collection period ends when consent is withdrawn.

If a participant permanently discontinues or temporarily discontinues study intervention because of an AE or SAE, the AE or SAE must be recorded on the CRF and the SAE reported using the Electronic Data Collection Tool.

Investigators are not obligated to actively seek information on AEs or SAEs after the close of the study. However, if the investigator learns of any SAE, including death, at any time after the active collection period, and the investigator considers the event to be reasonably related to the study intervention, the investigator must immediately upon awareness and under no circumstance should this exceed 24 hours, report the SAE using the SAE report form included in (APPENDIX 8).

7.4. Reporting SAEs the Sponsor Institution

All SAEs occurring in a participant during the active collection period are reported to CRO immediately upon awareness and under no circumstance should this exceed 24 hours, as indicated in APPENDIX 8. The investigator will submit any updated SAE data to the sponsor within 24 hours of it being available.

All serious adverse events must be reported by fax within 24 h from awareness of the event to MFAR Clinical Research, S.L. by fax (+ 34 93 253 11 68) or e-mail to investigacion@mfar.net.

7.5. Recording Non Serious AEs and SAEs on the CRF

All non serious AEs and SAEs occurring in a participant during the active collection period which begins after obtaining informed consent, will be recorded on the AE section of the CRF.

The investigator is to record on the CRF all directly observed, and all spontaneously reported AEs and SAEs reported by the participant.

If a participant begins a new anticancer therapy, the recording period for non serious AEs ends at the time the new treatment is started; however, SAEs must continue to be recorded on the CRF during the above-indicated active collection period. Note that a switch to a commercially available version of the study intervention is considered as a new anticancer therapy for the purposes of SAE reporting.

7.6. Follow-Up of AEs and SAEs

After the initial AE or SAE report, the investigator is required to proactively follow each participant at subsequent visits/contacts. For each event, the investigator must pursue and obtain adequate information until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 5.14).

In general, follow-up information will include a description of the event in sufficient detail to allow for a complete medical assessment of the case and independent determination of possible causality. Any information relevant to the event, such as concomitant medications and illnesses, must be provided. In the case of a participant death, a summary of available autopsy findings must be submitted as soon as possible to the CRO.

Further information on follow-up procedures is given in APPENDIX 3.

7.7. Regulatory Reporting Requirements for SAEs

Prompt notification by the investigator to the sponsor of an SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of a study intervention under clinical investigation are met.

The sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of a study intervention under clinical investigation. The sponsor will comply

with country-specific regulatory requirements relating to safety reporting to the regulatory authority, IRBs/ECs, and investigators.

Investigator safety reports must be prepared for SUSARs according to local regulatory requirements and sponsor policy and forwarded to investigators as necessary.

An investigator who receives SUSARs or other specific safety information (eg, summary or listing of SAEs) from the sponsor will review and then file it along with the SRSD(s) for the study and will notify the IRB/EC, if appropriate according to local requirements.

7.8. Adverse Event of Special Interest (AESI)

An Adverse Event of Special Interest is defined as any AE (serious or non-serious) that is of scientific and medical concern specific to the study treatment, for which ongoing monitoring and rapid communication to the Sponsor is required.

An Adverse Event of Special Interest (AESI) is defined as a noteworthy event for a product or class of products that the sponsor may wish to monitor carefully that are documented in a protocol or protocol amendment and may have special reporting requirements. The events may be serious or non-serious and could include events that might be potential precursors or prodromes for more serious medical conditions in susceptible individuals.

AESIs for elranatamab should be reported as an AE or SAE following the procedures described in Section 7.12.

Adverse Events of Special Interest (AESI) for elranatamab include the following:

CRS is a known toxicity of therapeutics that function by activation of immune effector cells. CRS is defined as a supraphysiologic response following any immune therapy that results in the activation or engagement of endogenous or infused T-cells and/or other immune effector cells. Symptoms can be progressive, must include fever at the onset, and may include hypotension, capillary leakage causing hypoxia and end organ dysfunction. Symptoms associated with CRS vary greatly and may be difficult to distinguish from other conditions. The severity of symptoms can be mild to life threatening, thus there should be a high index of suspicion for CRS if these symptoms occur. The severity of CRS will be assessed according to the consensus grading from the ASTCT. See APPENDIX 12.

For both the priming doses and first full dose (76 mg), premedication for CRS is required (see Section 5.2).

ICANS is a known toxicity of therapeutics that function by activation of immune effector cells. ICANS is defined as "a disorder characterized by a pathologic process involving the central nervous system following any immune therapy that results in the activation or engagement of endogenous or infused T-cells and/or other immune effector cells. Symptoms or signs can be progressive and may include aphasia, altered level of consciousness, impairment of cognitive skills, motor weakness, seizures, and cerebral edema" [42]. It has been observed following administration of some CAR-T-cells and BsAbs and can occur independently of CRS. The severity of ICANS will be graded according to the ASTCT consensus criteria. See APPENDIX 12.

Peripheral neuropathy is a common complication of MM and its treatment. Peripheral neuropathy can be caused by MM itself, either by the paraneoplastic effects of the monoclonal protein (polyneuropathy is an essential feature of POEMS syndrome) or in the form of radiculopathy from direct compression, and particularly by certain therapies, including IMiDs and PIs. Symptoms are usually symmetric and include paresthesia, numbness, burning sensation and muscle weakness; these are generally mild, but in rare cases can be disabling or even life-threatening. Treatment-emergent peripheral neuropathy symptoms are usually symmetric, distal and progressive [43]. Recently, peripheral neuropathy has been described following administration of BCMA-directed bispecific T-cell engagers [44].

Peripheral neuropathy (including GBS) is considered an AESI for elranatamab.

Work-up for new or worsening Grade ≥2 peripheral neuropathy should include a neurology consult, imaging (eg, MRI of the spine), NCV/EMGs, and lumbar puncture to assess CSF. In consultation with a neurologist, appropriate therapy for peripheral neuropathy (eg, steroids and/or IV immunoglobulin) should be considered.

Closely monitor participants for signs and symptoms of neuropathy following infections or following the administration of any vaccine.

AESIs are examined as part of routine safety data review procedures throughout the clinical trial and as part of signal detection processes.

All AESIs must be reported as an AE or SAE following the procedures described in Section 7.

The list of events of special interest will be available to the researchers and study monitors, through the Sponsor.

Severity of all AESIs will be graded using National Cancer Institute-Common Toxicity Criteria for Adverse Events (CTCAE, v5.0). Guidelines for dose modifications and interruptions for management of common toxicities associated with the study treatment(s) are provided in Section 5.9.

AESIs should be reported as an AE or SAE following the procedures described in Section 7.12.

7.9. Special Reporting Situations (SRS):

Special reporting situations (SRS) should be recorded in the eCRF. Any special reporting situation that meets the criteria of a serious adverse event should be reported according to the serious adverse event reporting timelines.

7.10. Assessment of Adverse Events

Each AE will be assessed by the investigator for severity and for a causal relationship with the study treatment as outlined below.

7.10.1. Severity Assessment

All AEs will be assessed by the Investigator for severity according to Common Terminology Criteria for Adverse Events (CTCAE) v5.0. The CTCAE severity grades 1 through 5 provide unique clinical descriptions of severity of each adverse event. The CTCAE v5.0 is available on the NCI/NIH website.

Please note that there is a distinction between **serious** and **severe** AEs: Severity is a measure of intensity whereas seriousness is defined by the criteria in Section 9. For example, a mild degree of gastrointestinal bleeding requiring an overnight hospitalization for monitoring purposes may be considered an SAE but is not necessarily severe.

7.10.2. Relationship to Study Drug

The Investigator must provide a causality assessment regarding the relationship of the event with the study drug for all AEs. One of the following categories should be selected based on medical judgment, considering all contributing factors:

- **Related**: A causal relationship between the medicinal product and AE is a reasonable possibility. For example, the occurrence of the AE cannot be explained by other causative factors. The AE, however, can be explained by pharmacological effect of the medicinal product such as a similar event having been reported previously, alteration of the dose effect, or the timing or seriousness of the AE, etc. Positive rechallenge/dechallenge is supportive.
- **Not Related**: A causal relationship between the medicinal product AE is not a reasonable possibility: there is no temporal relationship between the medicinal product and event, or an alternative etiology is more reasonable.

7.11. Collection and Recording of Adverse Events

AEs may be volunteered spontaneously reported by the study patient, or discovered by the study staff during physical examinations or by asking an open, non leading question such as, “How have you been feeling since your last study visit?” The Investigator will document the nature of AE, date of onset of the AE (and time, if known), date of outcome of the AE (and time, if known), severity of the AE, action taken with study drug as a result of the AE, assessment of the seriousness of the AE, and assessment of the causal relationship of the AE to study drug and/or study procedure.

All AEs (non-serious related events and related and unrelated serious events) will be recorded in the CRF over the course of the trial. For AE reporting, the verbatim used in the eCRF by investigators to identify AEs will be coded using the latest version of MedDRA coding dictionary. Description of individual underlying signs and symptoms should be avoided. When a clear diagnosis cannot be identified, each sign or symptom should be recorded as a separate AE.

Concomitant illnesses that existed before entry into the study are to be documented as medical history and will not be considered AEs unless the illness worsens after initiating protocol therapy.

All SAEs will be collected from the signing of the ICF and must be throughout the study and for at least 90 days after the last dose of study treatment. SAEs considered by the Investigator to be related

to study medication will be reported regardless of the timeframe from the last dose of study treatment. Disease progression is an efficacy criterion and is therefore not considered an AE or SAE (even if fatal). Disease progression should be documented but not reported as an SAE. If AEs/SAEs occur in relation to disease progression that are not consistent with the natural progression of the patient's disease, these AEs/SAEs must be reported per AE/SAE reporting requirements.

7.12. Time Period and Frequency for Collecting AE and SAE Information

The event will be reported by the sponsor via the CRO within the timeframes established by current Spanish legislation.

The reference safety information document in which to attribute the expected or unexpected nature of serious adverse reactions, for the purposes of expediting reporting, will be the Investigator's Brochures.

- All SAEs will be collected from the signing of the ICF until 90 days after discontinuing all study interventions, regardless of initiation of a new cancer therapy or transfer to hospice, at the time points specified in the SoA (Section 6). SAEs considered by the Investigator to be related to study medication will be reported regardless of the timeframe from the last dose of protocol therapy.

  However, any SAEs assessed as related to study participation (e.g., study intervention, protocol-mandated procedures, invasive tests, or change in existing therapy) or related to a product will be recorded from the time a participant consents to participate in the study up to and including any follow up.
- All AEs will be collected from the start of treatment until 90 days following discontinuation of study treatment regardless of initiation of a new cancer therapy or transfer to hospice at the time points specified in the SoA.
- Medical occurrences that begin before the start of study intervention but after obtaining informed consent will be recorded in the Medical History/Current Medical Conditions section of the eCRF.
- Pharmacovigilance responsibilities have been delegated by the Sponsor to the CRO. Therefore, CRO´s pharmacovigilance department will therefore assume these tasks.
- All SAEs will be recorded and reported to the CRO (which acts as sponsor´s designee) immediately and under no circumstance should this exceed 24 hours, as indicated in Section

7. The investigator will submit any updated SAE data to the CRO within 24 hours of it being available.

- Investigators are not obligated to actively seek AEs or SAEs after the conclusion of the study participation. However, if the investigator learns of any SAE, including a death, at any time after a participant has been discharged from the study, and he/she considers the event to be reasonably related to the study intervention or study participation, the investigator must notify to the CRO using the SAE report form included in APPENDIX 8).
- The method of recording, evaluating, and assessing causality of AEs and SAEs and the procedures for completing and transmitting SAE reports are provided in Section 7.

7.13. Follow-Up of Adverse Events

All AEs experienced by a patient, regardless of the suspected causality, will be monitored until the AE or SAE has resolved, until any abnormal laboratory values have returned to baseline or normal levels, until stabilized with a satisfactory explanation for the changes observed, until the patient is lost to follow-up, or until the patient has died.

7.14. Disease-Related Events and/or Disease-Related Outcomes Not Qualifying as SAEs

Disease progression does not need to be reported as a serious adverse event (SAE). However, if the underlying disease (i.e., progression) is greater than that which would normally be expected for the participant, or if the investigator considers that there was a causal relationship between treatment with study treatment(s) or protocol design or procedures and the disease progression, then this must be reported as an SAE. Death due to disease progression within the SAE reporting period, must be immediately reported of becoming aware. Death due to the disease under study is to be recorded on the Death eCRF.

7.15. Treatment of Overdose

7.15.1. Elranatamab

No specific treatment is recommended for an overdose of elranatamab.

In the event of an overdose of elranatamab, the investigator must:

- Contact the Clinical Coordinators immediately.
- Monitor the participant closely for AEs/SAEs and laboratory abnormalities until elranatamab can no longer be detected systemically (at least 3 months).
- Document the quantity of the excess dose as well as the duration of the overdosing in the eCRF.
- Overdose is reportable to the Sponsor only when associated with an SAE.
- Decisions regarding dose interruptions or modifications will be made by the investigator in consultation with the Clinical Coordinators based on the clinical evaluation of the participant.
- Overdose is reportable to the Sponsor only when associated with an SAE.
- Obtain a blood sample for PK analysis within 28 days from the date of the last dose of elranatamab if requested by the medical monitor (determined on a case-by-case basis).

7.16. Reporting to the Sponsor Institution

All SAEs and serious AESIs must be reported to CRO within 24 hours of becoming aware of the initial SAE/AESI or any follow-up information regarding the SAE/AESI using the SAE reporting information below. SAEs/AESIs must be reported after study completion, if the SAE/AESI is assessed as study-drug related.

Table 6 CRO SAE, Pregnancy, AESI Reporting Information

CRO SAE, Pregnancy, AESI Reporting Information
MFAR Clinical Research Representative: Federico Nepote Mailing address of the CRO: C/ Sinfonía 28, 2-1, 28001, Madrid, Spain Tel.: +34 93 434 44 12 Fax: +34 93 253 11 68 Email: investigacion@mfar.net

7.17. Pregnancy

Investigators should follow the protocol for guidance on subject education and ensure that all subjects adhere to the pregnancy prevention program.

All pregnancies or suspected pregnancies occurring in either a female subject of childbearing potential or partner of childbearing potential of a male subject are immediately reportable events.

Further describe the requirements, timelines and format to report pregnancy and pregnancy related events for the study.

The exposure of any pregnant female (eg, caregiver, pharmacist, study coordinator or monitor) is also an immediately reportable event.

7.17.1. Females of Childbearing Potential:

Pregnancies and suspected pregnancies (including elevated βhCG or positive pregnancy test in a female subject of childbearing potential regardless of age or disease state) occurring while the subject is on IP, or within 30 days of the subject's last dose of IP, are considered immediately reportable events. IP is to be discontinued immediately and the subject instructed to return any unused portion of the IP to the Investigator. The pregnancy/ suspected pregnancy must be reported to CRO immediately (within 24 hours of their knowledge of the event) by email, fax or phone and by sending a completed Pregnancy Report Form or approved equivalent Form to CRO.

The exposure of any pregnant female (e.g., caregiver or pharmacist) is also an immediately reportable event.

The female subject should be referred to an obstetrician-gynecologist, preferably one experienced in reproductive toxicity, for further evaluation and counseling.

The Investigator will follow the female subject until completion of the pregnancy, and must notify to CRO immediately about the outcome of the pregnancy (either normal or abnormal outcome) by email, fax or phone and by sending a completed Pregnancy Report Form or approved equivalent Form to PETHEMA Foundation through CRO.

Abnormal pregnancy outcomes (eg, spontaneous abortion, stillbirth, and congenital anomaly) are considered serious adverse events and must be reported using the Serious Adverse Event Form. It must be reported as an SAE to CRO within 24 hours of the Investigator's knowledge of the event using the SAE Report Form, or approved equivalent form.

All neonatal deaths that occur within 28 days of birth should be reported, without regard to causality, as SAEs. In addition, any infant death after 28 days that the Investigator suspects is related to the in utero exposure to the IP should also be reported to CRO as an SAE within 24 hours of the Investigator's knowledge of the event using the SAE Report Form, or approved equivalent form.

Follow-up information regarding the outcome of the pregnancy and any postnatal sequelae in the infant will be required.

Any female participant who becomes pregnant while participating will discontinue study intervention.

7.17.2. Male Subjects:

The effect of the study drug on sperm is unknown. Male subjects must not donate semen or sperm while receiving elranatamab, during dose interruptions or for at least 90 days following the last dose of elranatamab.

If a female partner of a male subject taking investigational product becomes pregnant, the male subject taking IP should notify the Investigator, and the pregnant female partner should be advised to call their healthcare provider immediately.

7.18. Reporting Product Complaints

A product quality complaint (PQC) is defined as any suspicion of a product defect related to manufacturing, labeling, or packaging, ie, any dissatisfaction relative to the identity, quality, durability, or reliability of a product, including its labeling or package integrity. A PQC may have an effect on the safety and efficacy of the product; therefore, the timely, accurate, and complete reporting and analysis of PQC information are crucial for the protection of subjects, investigators, and the Sponsor.

Any written, electronic or oral communication that alleges dissatisfaction related to manufactured clinical drug product with regards to its manufacturing, testing, packaging, or shipping, must be reported by the CRO on behalf of the supplier to within 1 working day of first becoming aware of the possible defect to carmen@fundacionpethema.es and investigacon@mfar.net. The product and packaging components in question, if available, must be stored in a secure area under specified storage conditions until it is determined whether the product is required to be returned for investigation of the defect. If the product complaint is associated with an SAE, the SAE must be reported separately in accordance with the protocol, and the SAE report should mention the product quality complaint.

8. DATA REPORTING

8.1. Data Submission

Sponsor and CRO will be responsible for processing and quality control of the data. Data will be handled in accordance with the Data Management Plan, Standard Operating Procedures and applicable regulatory guidelines.

Data management based on GCP refers to activities defined to achieve safe routines for efficient entry of subject information into a database, avoiding errors. The routines include procedures for handling of eCRFs, database set-up and management, data entry and verification, data validation, quality control (QC) of database, and documentation of the performed activities, including information of discrepancies in the process. The database, the data entry screens and the program, will be designed in accordance with the clinical study protocol by the Sponsor.

All participant data relating to the study will be recorded on printed or electronic CRF unless transmitted to the sponsor or designee electronically (e.g., laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the eCRF.

The eCRFs should always reflect the latest observations on the subjects participating in the study. Therefore, the eCRFs should be completed as soon as possible during or after the subject's visit. Each study site Investigator must verify that all data entries in the eCRFs are accurate and correct.

If some assessments are not done, or if certain information is not available, not applicable or unknown, the Investigator or authorized designee should indicate this in the eCRF.

- The investigator must maintain accurate documentation (source data) that supports the information entered in the eCRF.
- The investigator must permit study-related monitoring, audits, IRB/IEC review, and regulatory agency inspections and provide direct access to source data documents.
- The sponsor or designee is responsible for the data management of this study including quality checking of the data.
- Study monitors will perform ongoing source data verification to confirm that data entered into the eCRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is

being conducted in accordance with the currently approved protocol and any other study agreements, ICH GCP, and all applicable regulatory requirements.

- Records and documents, including signed ICF, pertaining to the conduct of this study must be retained by the investigator for 25 years from the issue of the final Clinical Study Report (CSR)/equivalent summary unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the sponsor and prior Sponsor approval.

8.2. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator's site file.

- Data entered in the eCRF that are transcribed from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

The eCRF is essentially considered a data entry form and should not constitute the original (or source) medical records unless otherwise specified.

Source documents are all documents used by the Investigator or hospital that relate to the subject's medical history, that verify the existence of the subject, the inclusion and exclusion criteria, and all records covering the subject's participation in the study.

They include medical records, subject files, laboratory and medical test results and the essential documents for the clinical trials: screening, enrolment log, material dispensing records, etc.

Each Investigator is responsible for maintaining source documents. These will be made available for inspection by the study monitor at each monitoring visit. All supportive documentation submitted with the eCRF, such as laboratory data should be clearly identified with the study, visit and subject number. Any personal information (e.g., subject name, initials) should be removed or rendered illegible to preserve individual confidentiality.

8.3. Web-based eCRF

Data collection for this study will consist of electronic data capture for all eCRF information. Sponsor will supply the eCRF. All study site Investigators agree to maintain accurate eCRFs and source documentation as part of the case histories. All information is to be filled in the subject's eCRF. If an enrolled subject is screening failure, only minimum data (such as demographics and consent date) and the reason for failing screening should be reported on the eCRF. In general, no queries for missing data on these subjects will be issued for procedures indicated as 'Not done'.

For entered subjects, information captured on the source documents will be entered into the subject's eCRF by study site personnel and monitored at the study site. If an item is not available or is not applicable, this fact should be indicated by a missing reason.

Each completed eCRF must be reviewed, signed, and dated by the study site Investigator in a timely manner. The completed eCRF will be reviewed by the study Monitor as soon as practical after completion. A copy of the final, approved and signed eCRF will be provided to the site and should be stored in the appropriate files.

Clinical data (including AEs and concomitant medications) will be entered into an Electronic Data Capture (EDC) application. The data system is password protected and includes internal quality checks, such as automatic range checks, to identify data that appear inconsistent, incomplete or inaccurate. Clinical data will be entered directly from the source documents. Roles and rights of the study site personnel responsible for entering the study data into the eCRF will be determined in advance. Only authorized study site personnel designated by each study site Investigator will complete data collection.

8.4. Data and Safety Monitoring

The clinical monitors are employees of CRO, and representatives of the sponsor. As such, they have the obligation to follow the trial closely so that all aspects of the trial are carefully monitored for compliance with applicable government regulations and with ICH E6(R2) guidelines.

The clinical monitors will visit the study sites and Investigators at intervals as defined in the monitoring plan, in addition to maintaining necessary contact through telephone, e-mail, and letter. The clinical monitors will maintain current personal knowledge of the trial through observation, review

of trial records and source documentation, and discussion of the conduct of the trial with the study site Investigators and staff.

Additionally, safety data will be independently analyzed by the Data Safety Independent Committee (DSMC) and will recommend how to proceed with the study. The DSMC will meet periodically as scheduled in APPENDIX 14.

8.5. The Query Process

Each monitor will review the eCRFs and evaluate them for completeness and consistency. Each eCRF will be compared with the respective source documents to ensure that there are no discrepancies between critical data. All entries, corrections and alterations are to be made by the Investigator or his/her authorized designee. The monitor cannot enter data in the eCRFs.

The appropriate investigational staff will answer queries generated in the application. This process is audit trailed meaning that the name of investigational staff, time, and date is logged.

8.6. Audits

The sponsor may choose to schedule and conduct periodic audits of ongoing clinical studies. Audits are independent of, and separate from, routine monitoring or quality control functions and are conducted to assure accuracy and compliance with the protocol, governing regulatory authorities and/or ICH E6(R2) guidelines.

The Investigators and institutions involved in the trial will permit trial-related monitoring, audits, IEC review, and regulatory inspection(s) by providing direct access to all trial records. In the event of an audit, the Investigator agrees to allow the sponsor, representatives of the sponsor, and the governing regulatory agency access to all trial records.

The Investigator should immediately notify the sponsor of any inspections scheduled by any regulatory authorities and promptly forward copies of any inspection reports received to the sponsor.

8.7. Protocol Deviations

Protocol deviations are unintended and/or unanticipated departures from the procedures and/or processes in the protocol as approved by the sponsor, the CA and IEC, and agreed to by the Investigator.

Minor deviations usually have a limited impact on study outcomes and do not involve ICH E6 guidelines, inclusion/exclusion criteria or primary endpoint criteria.

Major deviations are those that may adversely affect the rights, safety or welfare of subjects and / or quality and integrity of data.

Each study site Investigator must document and explain in the subject's source documentation any deviations from the approved protocol. Investigators may implement a deviation from the protocol to eliminate an immediate hazard to trial subjects without prior IEC informed consent approval, but the deviation must be reported to the monitor/CRA within 1 working day. Such incidents will be evaluated for potential safety hazards of the ongoing study, and if deemed appropriate, a protocol amendment will be issued.

The monitor/CRA will document protocol deviations throughout the course of monitoring visits. The monitor will notify the Investigator during a visit and a "Protocol Deviation Form" is completed and signed by the investigator and by the monitor.

8.8. Trial Record Retention

Essential documents and eCRF data should be retained during 25 years.

The investigator will maintain all study records in accordance with GCP guidelines as established by ICH and corresponding regulatory requirements.

8.9. Study and Site Closure

Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time, provided there is reasonable cause and sufficient notice is given in advance to the Sponsor and CRO of the intended termination.

Reasons for the early closure of a study site by the sponsor or investigator may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IRB/IEC or local health authorities, the sponsor's procedures, or GCP guidelines.
- Inadequate recruitment of participants by the investigator.
- Discontinuation of further study intervention development.

9. Statistical Methods

9.1. Study Design

This is a Phase II, open-label, single-arm, multicenter study to evaluate the efficacy and safety of elranatamab monotherapy at the dose of 76 mg subcutaneously in participants with RRMM who had received prior treatment with immunomodulatory drugs, protease inhibitors, and anti-CD38 therapy and were refractory to the last line of therapy, defined as progression while receiving treatment or in the first 60 days after the last dose of treatment.

The study consists of a screening/baseline period, a treatment period, and a posttreatment follow-up period.

This is a pilot study aiming to evaluate the efficacy as well as the safety of elranatamab in a population of relapsed multiple myeloma patients three drug class exposed after 1 or 2 prior lines of therapy and refractory to the last line of therapy. Therefore, no formal hypothesis testing will be conducted. The sample size is selected to allow for collection of sufficient data for evaluating the efficacy and safety and tolerability of elranatamab. Accordingly, 50 patients with myeloma will be included.

9.2. Endpoints: definitions and analysis

9.2.1. Primary Efficacy Endpoints and Secondary Time-to Event Endpoints:

Primary endpoint is to determine the efficacy of elranatamab monotherapy in terms of undetectable measurable residual disease at 6 and 12 months. MRD negativity rate, defined as the percentage of participants who are MRD negative by Next generation flow cytometry (NGF) method and with a sensitivity level of at least 10^{-5}.

Other efficacy endpoints include:

- Sustained undetectable Measurable Residual Disease annually rate as per International Myeloma Working Group (IMWG) criteria evaluated by the investigators
- Overall response rate (ORR). ORR, defined as the percentage of participants with a confirmed partial response (PR) or better per IMWG
- Duration of response (DoR). DoR, defined as the time from first documented evidence of PR or better until progressive disease (PD) per IMWG or death due to PD among participants who achieve confirmed PR or better

- Time to response (TTR). TTR, defined as the time between the date of randomization and the first documented evidence of response (PR or better) among participants who achieve confirmed PR or better.
- Progression-free survival: PFS defined as the time from the date of study inclusion until confirmed PD based on investigator's assessment per IMWG response criteria or death due to any cause, whichever occurs first. PFS data will be censored at the last date on which it was known that the patient did not exhibit disease progression in 1) patients whose disease has not progressed at the time of analysis and 2) patients who have withdrawn from the treatment phase of the study before progression was documented (including those who died for reasons unrelated to MM).
- Overall survival (OS). OS, defined as the time from study inclusion until death due to any cause Time (in months) from the date of inclusion in the study until the date of death. Data from patients lost to follow-up will be censured at the date of the last visit.
- PFS and OS will be assessed in those patients who stop treatment because of sustained MRD-ve for 12 months.

9.2.2. Secondary Safety Endpoints:

- Number of participants with adverse events (AEs) during the treatment period

  An AE is any untoward medical occurrence in a clinical study participant, temporally associated with the use of a study treatment, whether or not considered related to the study treatment. A SAE is defined as any untoward medical occurrence that at any dose may result in death, is life-threatening, requires inpatient hospitalization or prolongation of existing hospitalization, results in persistent disability/incapacity, is a congenital anomaly or birth defect and important medical events may jeopardize the participant or may require medical or surgical intervention to prevent one of the other outcomes listed before.

All AEs that occur during the study will be included in data lists that will be organized by patient. AEs that are the result of the study treatment will be included in a table. These AEs are those that: occur after administration of the first dose of study drug and for a period of four weeks after the last dose of the study drug is administered, are considered related to the study drug regardless of the date of onset, or any event present during the screening visit and which worsens in severity, or which the investigator later considers to be related to any of the study drugs. Those events which are

considered to be related to the treatment (possibly, probably, or definitely related) will also be included in a table. As well, a table detailing adverse events in order of maximum grade of severity will also be provided. Deaths, serious adverse events, and adverse events that lead to withdrawal from the study will be presented in a separate table.

The changes observed in vital signs will be summarized in a similar manner to analytical parameters, and any abnormal value will be included in a table.

- Incidence and severity of Cytokine Release Syndrome (CRS) and Immune effector cell associated neurotoxicity syndrome (ICANS) according to the American Society for Transplantation and Cellular Therapy (ASTCT) criteria
- Incidence and severity of other neurotoxicities.
- Incidence of cytopenias and infections.

9.2.3. Secondary Biological Endpoints:

- Circulating Tumor Cells at baseline to evaluate its prognostic value.
- Measurement of serum BCMA levels and its correlation with response.
- Gene expression techniques (cytogenetic abnormalities and RNA sequencing and single cell sequencing studies) coupled with intelligent clinical and molecular data analysis (i.e. machine learning) to identify factors that could predict response to elranatamab.
- Mass spectrometry to complement the evaluation of the response by the conventional techniques like electrophoresis, immunofixation and next generation flow.
- Identification and characterization of T, B, NK, monocytes and normal/clonal PC (including BCMA expression).
- Quantification of soluble factors: IL6, IL2, IFNγ and TNFα.
- Fluorescence-activated cell sorting (FACS) sorting of T and Pathological Plasma Cells at inclusion and at PD, and of T cells after the first dose.

9.3. Descriptive analyses

Descriptive analyses will consist of evaluating the changes that have occurred from the time patients attend the screening visit in the following values that are of significance in myeloma:

- Monoclonal protein in serum and/or urine.
- sFLC assay
- Percentage of PC in BM aspiration, determined by morphological analysis and flow cytometry.

These variables can be analyzed in relation to the changes that have occurred since the screening visit and successive measurement intervals.

9.4. End points and long-term observation

A participant is considered to have completed the study if he or she is followed until disease progression, withdrawal of consent, loss to follow up, death or end of study.

Completion of the study by all patients included in the trial, performance of all planned activities (SoA, Section 6) and solving of all inconsistencies is constituted as the end of the trial. Sample size, Accrual Rate and Study Duration

9.5. Descriptive analysis

All variables will be described by using summary statistics such as counts, mean, standard deviation, median, minimum, maximum and 25th and 75th percentiles for continuous and counts and percentages for categorical variables. The 95% confidence intervals for the mean and the proportions and free distribution confidence intervals for the median will be calculated. The graphical analysis will be carried out using bar diagrams, scatter diagrams, box diagrams, profiles and others when appropriate. Variables of classification (as for example phase, frail / non-frail) will be used when appropriate.

9.6. Analysis of Endpoints

The details of statistical analyses can be found in the document Trial Statistical Analysis Plan.

9.6.1. Analysis of Primary and Secondary Efficacy Endpoints

- Minimal Residual Disease (MRD) Negativity

MRD negativity defined at selected sensitivity level (e.g. 10^{-5} or 3 x 10^{-6}) in bone marrow will be analyzed. A sensitivity of at least 10^{-5} will be used for across study comparisons. MRD will be summarized with frequency tabulations by cohorts. MRD may be summarized by treatment cycle/time or phase, and for difference response status, based on data availability. The proportion of patients reporting negative MRD at months 6, 12 and yearly thereafter will be assessed computing the 95% confidence interval following the recommended method. The analysis of this primary endpoint will be

done considering as denominator the evaluable population for MRD analysis including those patients at risk at the specified time point, with BM done and all patients who had early discontinued because of progressive disease will be considered as MRD+ve.

9.6.2. Analysis of Secondary Endpoints

- Response Rate

The proportion of patients across different phases of study (month 6 and 12) reporting overall response and categories (Complete Response rate, stringent complete response, …) will be evaluated computing the 95% confidence interval following the recommended method. The number of patients responding will be divided by the number of participants in the ITT data set. Responses that are documented after the subjects receive any subsequent anti myeloma treatment will not be counted in the numerator as responses. However, these subjects will be included in the denominator per ITT population. Within the group of patients in CR, the patients who achieved CR with positive immunofixation (CR.IF+) will be differentiated from those who achieved CR.IF- with the corresponding 95% confidence interval.

- Progression-free Survival

Progression-free survival (PFS) will be calculated as the time between the study start and disease progression determined by the investigators, or death with any cause, whichever occurred earlier.

The analysis will be based on European Medicines Agency (EMA) censoring rules (EMA, 2013). Subject who neither progressed nor died will be censored on the date of their last adequate response assessment. Subjects without valid any post-baseline response assessment will be censored on their randomization date.

Progression free survival will be assessed using the Kaplan-Meier product-limit survivor-function estimate for overall population and frail and non-frail populations.

- Overall Survival

Overall survival will be assessed using the Kaplan-Meier product-limit survivor-function estimate as in progression free survival. Overall survival is defined as the time between study start and death. Subjects who die before or on the date of data cutoff will be considered to have had an OS event. Subjects who do not have a death record prior to or on the cutoff date will be censored at the “last date known alive”.

- Changes in immune profiling

Components from immune profiling will be evaluated by means of patient profiles and comparing the values against normal ranges. Differences of mean values between patients reporting remission, disease progression, etc. will be estimated using 95% recommended confidence intervals.

- Adverse events

The proportion of patients across different phases of study reporting at least one AE will be evaluated computing the 95% confidence interval following the recommended method. All AE characteristics (AEs that are the result of the study treatment, related to the treatment (possibly, probably, or definitely related), grade of severity, deaths, serious adverse events (by SOC), that leading to withdrawal from the study) will be presented in a separate table. The changes observed in vital signs will be summarized and tabulated.

- Change from baseline in laboratory parameters

Hematology, chemistry and clinical parameters will be evaluated by means of patient profiles and comparing the values against normal ranges.

9.6.3. Analysis Populations

Following study populations will be defined:

- **Intention-to-treat population (ITT)** defined as data set including all enrolled patients (with valid informed consent and not screen failure) who begin treatment in the clinical trial. Analysis of the primary efficacy endpoint will be carried out in this population but also in the evaluable population for this primary endpoint.
- **Evaluable population (EP)** defined as data set including all patients that meet the basic protocol criteria and have been evaluated after receiving elranatamab, in order to evaluate the secondary endpoint. The safety analysis for treatment administration will include all patients that have received at least one dose of the study treatment.
- **Safety population (SP)** defined as data set including all subjects who receive at least one dose of study treatment.

9.7. Demographics and baseline characteristics

The demographics and baseline characteristics will be summarized overall using the ITT population. Individual subject listings will be provided to support the summary tables.

A summary of medical and surgical history, and summary tables of concomitant medications will be provided by treatment arms and overall, for each treatment phase.

9.8. Procedures for processing missing, unused, and spurious data

All available safety and efficacy data will be included in data lists and tables. Values will not be imputed to missing data. Patients who have received the study drug and have not undergone safety-related follow-up will not be included in the safety analyses since inclusion of these patients would lower the percentages of patients with adverse events or laboratory toxicities. Possible confusing or erroneous data will be examined in accordance with standardized data control procedures. Any spurious or erroneous data will be examined in according with standardized data management procedures.

9.9. Procedures for reporting deviations from the original statistical plan

All deviations from the original statistical plan will be included in the final clinical trial report.

10. Ethical, Legal, and Administrative Aspects

10.1. Good Clinical Practice

The current clinical trial will be conducted in accordance with the protocol, the principles established in the current revised version of the Declaration of Helsinki (APPENDIX 1) and the applicable regulatory requirements, particularly) ICH: E6-R2: Guideline for good clinical practice - Step 5 (2017), the Regulation (EU) 536/2014 relative to clinical trials on medicinal products for human use and the locally applicable regulations (e.g., In Spain, the Royal Decree on Clinical Trials 1090/2015).

10.2. Informed Consent

An approved informed consent form will be provided by the sponsor to the investigative site. The written Informed Consent Form (ICF) should be prepared in the local language(s) of the potential subject population. In obtaining and documenting informed consent, the Investigator should comply with the applicable regulatory requirements, and should adhere to GCP and to the ethical principles that have their origin in the Declaration of Helsinki. The consent form and any revision(s) should be approved by the Ethics Committee prior to being provided to potential subjects.

Before a subject's participation in the study, it is the Principal Investigator's (or their designee) responsibility to obtain freely given consent in writing, from the subject after adequate explanation of the aims, methods, anticipated benefits, and potential risks of the study and before any protocol-specific screening procedures or any study drugs are administered. Subjects must have the opportunity to ask questions and receive answers and will have adequate time to decide whether or not to participate in the study. Once the Investigator is assured that the subject understands the implications of participating in the trial, the subject will be asked to give consent to participate in the trial by signing the informed consent. The ICF should be signed and personally dated by the subject and by the physician who conducted the informed consent discussion (Principal Investigator or designee). The subject's written informed consent should also be documented in the subject's medical records.

The Investigator shall provide a copy of the signed informed consent to the subject. A second original form shall be maintained in the Investigator study file at the site.

If the informed consent is revised during the course of the trial, all active participating subjects must sign the revised form approved by the Ethics Committee.

The subject participating in a clinical trial, or his/her legal representative, may withdraw consent at any time without giving any reason and without this involving any penalty or prejudice for the participating subject.

10.3. Protocol Approval and Amendment

ICH guidelines require that approval be obtained from Health Authorities and an Ethics Committee before human subjects can participate in research studies. Prior to the trial onset, the protocol, informed consent, advertisements to be used for subject recruitment (if applicable), and any other written information regarding this trial to be provided to the subject will be approved by the Ethics Committee. The clinical study will only be started when both the Health Authorities and an Ethics Committee have considered that the expected benefits for the trial subject and society justify the risks; in addition, the trial will only be continued if compliance with this criterion is constantly supervised.

The CRO will obtain Ethics Committee approvals on behalf of the sponsor and investigators. All regulatory approvals should be signed by the Ethics Committee Chairman or designee and must identify the Ethics Committee name and address, the clinical protocol by title and/or protocol number and the date approval and/or favorable opinion was granted. Documentation of all Health Authorities and Ethics Committee approvals and of the Ethics Committee compliance with ICH E6 (2) will be maintained by the site and will be available for review.

Any change in the approved protocol will require a Protocol amendment. The Investigator must not make any change in the study without favorable opinion from the Ethics Committee and authorization from the Health Authorities, except as necessary to eliminate an impending and obvious risk for the subjects except when necessary to remove an apparent, immediate hazard to subjects. Protocol changes introduced to eliminate an impending and obvious risk may be implemented immediately, but must subsequently be documented in an amendment, reported to the Ethics Committee and be submitted to the relevant Health Authorities within the required timeframe.

Any substantial amendments to the protocol must be submitted in writing to the Ethics Committee and the Health Authorities for approval before the changes proposed in the amendment are implemented. Depending on the magnitude of the change, the recruitment may be temporally halted.

The sponsor does not have to notify non-substantial amendments to the Health Authorities or the Ethics Committee. However, any non-substantial amendments will be recorded and contained in the documentation when it is subsequently submitted, for example in the subsequent notification of a substantial amendment. Documentation of any non-substantial amendments will be available on request for inspection at the trial site or the sponsor premises as appropriate.

10.4. Obligations of all participants in the trial

10.4.1. Investigator

The responsibilities of the investigator are specified in article 41 of Royal Decree 1090/2015. The investigator must be in agreement with this protocol and possess thorough knowledge of the properties of the products used in the clinical trial.

The investigator should give the patient information sheet to the patient and assist him or her in understanding the explanations provided therein. It is important that the investigator informs the patient that his or her participation in the study is completely voluntary, and that this will not affect the patient/physician relationship. The investigator should also guarantee the patient that all persons involved in the study will maintain the confidentiality of all patient information.

The principal investigator or one of his collaborators will be responsible for collecting, recording and reporting the data correctly, and will guarantee the immediate reporting of serious adverse or unexpected events within 24 hours to CRO (by mail at: investigacion@mfar.net or by fax at: +34 93 253 11 68).

The investigator is responsible for preparing and maintaining accurate, complete medical histories, which are designed to document all observations and other relevant study data relating to each participant in the trial. All information recorded in the CRFs for this study must be consistent with each patient’s original documentation.

It is the responsibility of the investigator to regularly report to the Clinical Research Ethics Committee on the progress of the study; the investigator will be jointly responsible along with the sponsor for producing the clinical trial report.

10.4.2. The trial monitor

The trial monitor will undertake to directly monitor the conduct of the trial, following Good Clinical Practice guidelines. The responsibilities of the monitor are specified in article 40 of Royal Decree 1090/2015.

All study data recorded in the original documents will be transcribed onto the CRFs. The study data will be entered into a validated and secure data processing system and back-up copies will be stored. Any changes to the study data will be recorded.

During the course of the trial, the monitor will visit the trial sites to verify compliance with the protocol, to compare the CRFs with the patients' individual medical histories, evaluate drug accountability and ensure that the study is being conducted in accordance with the pertinent regulatory requirements. The CRFs will be checked against the original documentation. Medical histories will be verified in such a way that guarantees patient confidentiality. Any instances of data omission or of data that cannot be interpreted will be handled by the investigator.

10.4.3. The sponsor

The obligations of the sponsor are specified in article 39 of Royal Decree 1090/2015.

The sponsor of the clinical trial is the physical or legal person who has an interest in the trial being conducted. The sponsor signs authorization requests addressed to the REB and/or the Spanish Medicine Agency (AEM) and takes responsibility for the trial, including its conduct, initiation, and end. The sponsor is also in charge of ensuring compliance with the pertinent legal regulations.

The PETHEMA Foundation assumes the obligations of the sponsor set out in Article 39 of Royal Decree 1090/2015, and has the means and collaborators necessary to fully guarantee the fulfillment of said obligations.

10.5. Subject Confidentiality and Data Protection

All laboratory specimens, evaluation forms, reports, and other records will be identified in a manner designed to maintain subject confidentiality. Each Investigator will ensure that all site personnel involved will respect the confidentiality of any information about trial subjects. Management of personal data from subjects participating in the trial, particularly as regards consent, will comply regulation (EU) 2016/679 of the European Parliament and of the Council of 27 April 2016 on the protection of natural persons with regard to the processing of personal data and on the free movement of such data, and repealing Directive 95/46/EC (General Data Protection Regulation) and the Spanish Organic Law 3/2018 of 5th December, on the protection of personal data and digital rights.

At each site, all records will be kept in a secure storage area with limited access. Clinical information will not be released without the written permission of the subject. Subject identity is confidential and may only be known by the Investigator, trial personnel, appointed auditors and monitors, and Health Authorities.

Each Investigator and all employees and coworkers involved with this trial may not disclose or use for any purpose other than performance of the trial, any data, record, or other unpublished, confidential information disclosed to those individuals for the purpose of the trial. Prior written agreement from the sponsor or its designee must be collected for the disclosure of any said confidential information to other parties.

10.6. Data Quality Assurance

All participant data relating to the study will be recorded on printed or electronic CRF unless transmitted to the sponsor or designee electronically (e.g., laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the eCRF.

- The investigator must maintain accurate documentation (source data) that supports the information entered in the eCRF.
- The investigator must permit study-related monitoring, audits, IRB/IEC review, and regulatory agency inspections and provide direct access to source data documents.

- The sponsor or designee is responsible for the data management of this study including quality checking of the data.
- Study monitors will perform ongoing source data verification to confirm that data entered into the eCRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being conducted in accordance with the currently approved protocol and any other study agreements, ICH GCP, and all applicable regulatory requirements.
- Records and documents, including signed ICF, pertaining to the conduct of this study must be retained by the investigator for 25 years from the issue of the final Clinical Study Report (CSR)/equivalent summary unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the Sponsor and prior approval of the Sponsor.

10.7. Access to Source Documents

- Source documents provide evidence for the existence of the patient and substantiate the integrity of the data collected. Source documents are filed at the investigator’s site.
- Data entered in the eCRF that are transcribed from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

10.8. Study Monitoring

The clinical monitors are employees of CRO, and representatives of the sponsor. As such, they have the obligation to follow the trial closely so that all aspects of the trial are carefully monitored for compliance with applicable government regulations and with ICH E6(R2) guidelines.

The clinical monitors will visit the study sites and Investigators at intervals as defined in the monitoring plan, in addition to maintaining necessary contact through telephone, e-mail, and letter. The clinical monitors will maintain current personal knowledge of the trial through observation, review

of trial records and source documentation, and discussion of the conduct of the trial with the study site Investigators and staff.

10.9. Audits and Inspections

The regulatory authorities, the REB and the sponsor or designated representative can request access to all original documents, case report forms and any other study documentation for the purpose of auditing or inspecting the site. The investigator must guarantee direct access to these documents and cooperate at all times in the execution of these activities.

Care will be taken during these procedures to ensure the protection of personal information, in accordance with data protection laws.

If requested, the investigator will grant monitor(s) and auditor(s) from the Sponsor or its designees and regulatory authority(ies) access to the patient's original medical records for verification of data gathered on the data capture records and to audit the data collection process. The patient's confidentiality will be maintained and will not be made publicly available to the extent permitted by the applicable laws and regulations.

10.10. Archival

The investigator will maintain all study records in accordance with Good Clinical Practice as set out in the International Conference on Harmonization and corresponding regulatory requirements.

10.11. Publications

Results of this clinical trial, positive or negative, will be presented at scientific conferences and published in scientific journals.

10.11.1. Basic trial guidelines

The principal investigators and/or the participating sites undertake not to use or transfer to third parties, nor distribute or publish the results of this trial without prior written consent from PETHEMA F, sole sponsor of this trial. The following conditions should be observed at all times:

a) The results of this trial shall not be published until the conclusion of the trial, or before if agreed by both parties.
b) The sponsor must not cite the names of the investigators without their permission, except in references to already published work.
c) The sponsor shall permit the publication of data obtained from this study in scientific journals of recognized prestige, and permit the data to be presented at seminars and conferences within the medical field, as long as the conditions set out in paragraph a) of this section are observed, and the sponsor is allowed to review the final draft of the article at least 4 weeks in advance.

It is expected that the results of this trial will be published in a scientific journal and as abstracts/presentations at medical conferences. Current guidelines and publishing recommendations of the ICMJE will be followed.

10.11.2. PETHEMA Foundation Group publication conditions

The following guidelines should be observed for any publications based on PETHEMA Foundation Group studies:

a) The names of participating sites will appear in all publications, and those individuals who have participated in the design, follow-up, results analysis and writing the publication will be listed as authors.
b) The persons responsible for the protocol will also be responsible for presenting the results at PETHEMA Foundation meetings and scientific conferences, as well as for writing papers based on the protocol. The persons responsible for the protocol will have assistance writing these papers if required.
c) The final draft of the text may be submitted for review and approval by another member of PETHEMA Foundation with proven expertise, so that his/her suggestions and changes can help to improve the quality of the work. This person can appear as co-author at the discretion of the persons in charge of the protocol. If the reviewer is not listed as a co-author, he/she should be mentioned in the acknowledgements section.
d) Inclusion of the data manager as a co-author is at the discretion of the persons in charge of the protocol. If the data manager is not listed as co-author, she/he will appear in the acknowledgements section.

e) The statistician (if applicable) can be listed as co-author or appear in the acknowledgements section depending on his/her degree of participation, and at the discretion of the persons responsible for the protocol.

f) The statistician and the data manager will not be included in the calculation when determining the maximum number of authors, based on the number of patients included per site.

g) A maximum number of authors will be determined per each participating site who will appear in the article heading. This selection, based on the number of included patients, will be made according to the criteria concerning patients who have contributed to the study (as long as evaluation and follow-up were carried out up to the final evaluation), and according to the following guidelines:

- Protocols that have included less than 200 patients: each site will have the right to one author per every 5% of patients included out of the total recruited for the study. There will be a limit of 15 authors for any publication. This restriction will be applied to presentations at scientific conferences.

h) If the conference organizers or journal specifies a limit to the number of authors, the number will be reduced starting with the last authors to be cited. The decision of those responsible for the protocol will prevail in the event of discrepancies

i) All institutions and persons that have participated in the protocol will appear in an appendix.

Before submitting the manuscript for publication, a copy will be sent to each author to make any changes they feel are appropriate. If no response is given within the maximum timeline set by the person in charge of the protocol, it will be assumed that the authors agree with the content of the manuscript. Given that there may be a considerable number of authors and that, on occasion, the suggestions may be redundant, contradictory, or minor, the writers of the draft publication will be responsible for producing the final version.

10.12. Insurance policy

The sponsor has contracted an insurance policy to cover the responsibilities of the investigator and other parties participating in the study, according to the applicable legislation.

- Insurance company: HDI

- Policy Number: 08054309-30149

10.13. Study and site closure

Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed. The investigator may initiate study-site closure at any time, provided there is reasonable cause and sufficient notice is given in advance of the intended termination.

Reasons for the early closure of a study site by the sponsor or investigator may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IRB/IEC or local health authorities, the sponsor's procedures, or GCP guidelines
- Inadequate recruitment of participants by the investigator
- Discontinuation of further study intervention development

For the study as a whole, the end of study is defined as the last patient of the last visit (LPLV), expected for 3Q 2029.

10.14. Early Study Termination

The clinical trial may be terminated prematurely or suspended at the request of Health Authorities or if new safety or efficacy information leads to an unfavorable risk benefit judgment for elranatamab. The Sponsor may discontinue the trial if it becomes unjustifiable for medical or ethical reasons, poor enrolment, or for safety reasons.

Health Authorities and Independent Ethics Committees (IECs)/Institutional Review Boards (IRBs) will be informed about the discontinuation of the trial in accordance with applicable regulations.

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APPENDIX 1 - DECLARATION OF HELSINKI

WMA Declaration of Helsinki - Ethical Principles for Medical Research Involving Human Subjects

Adopted by the
18th WMA General Assembly, Helsinki, Finland, June 1964
and amended by the
29th WMA General Assembly, Tokyo, Japan, October 1975
35th WMA General Assembly, Venice, Italy, October 1983
41st WMA General Assembly, Hong Kong, September 1989
48th General Assembly, Somerset West, Republic of South Africa, October 1996
52nd General Assembly, Edinburgh, Scotland, October 2000
Note of Clarification, added by the WMA General Assembly, Washington 2002
Note of Clarification, added by the WMA General Assembly, Tokyo 2004
59th General Assembly, Seoul, Republic of Korea, October 2008
64th General Assembly, Fortaleza, Brazil, October 2013

Preamble

1. The World Medical Association (WMA) has developed the Declaration of Helsinki as a statement of ethical principles for medical research involving human subjects, including research on identifiable human material and data.

The Declaration is intended to be read as a whole and each of its constituent paragraphs should be applied with consideration of all other relevant paragraphs.

2. Consistent with the mandate of the WMA, the Declaration is addressed primarily to physicians. The WMA encourages others who are involved in medical research involving human subjects to adopt these principles.

General Principles

3. The Declaration of Geneva of the WMA binds the physician with the words, “The health of my patient will be my first consideration,” and the International Code of Medical Ethics declares that, “A physician shall act in the patient's best interest when providing medical care.”

4. It is the duty of the physician to promote and safeguard the health, well-being and rights of patients, including those who are involved in medical research. The physician’s knowledge and conscience are dedicated to the fulfilment of this duty.

5. Medical progress is based on research that ultimately must include studies involving human subjects.

6. The primary purpose of medical research involving human subjects is to understand the causes, development and effects of diseases and improve preventive, diagnostic and therapeutic interventions (methods, procedures and treatments). Even the best proven interventions must be evaluated continually through research for their safety, effectiveness, efficiency, accessibility and quality.

7. Medical research is subject to ethical standards that promote and ensure respect for all human subjects and protect their health and rights.

8. While the primary purpose of medical research is to generate new knowledge, this goal can never take precedence over the rights and interests of individual research subjects.

9. It is the duty of physicians who are involved in medical research to protect the life, health, dignity, integrity, right to self-determination, privacy, and confidentiality of personal information of research subjects. The responsibility for the protection of research subjects must always rest with the physician or other health care professionals and never with the research subjects, even though they have given consent.

10. Physicians must consider the ethical, legal and regulatory norms and standards for research involving human subjects in their own countries as well as applicable international norms and

standards. No national or international ethical, legal or regulatory requirement should reduce or eliminate any of the protections for research subjects set forth in this Declaration.

11. Medical research should be conducted in a manner that minimises possible harm to the environment.

12. Medical research involving human subjects must be conducted only by individuals with the appropriate ethics and scientific education, training and qualifications. Research on patients or healthy volunteers requires the supervision of a competent and appropriately qualified physician or other health care professional.

13. Groups that are underrepresented in medical research should be provided appropriate access to participation in research.

14. Physicians who combine medical research with medical care should involve their patients in research only to the extent that this is justified by its potential preventive, diagnostic or therapeutic value and if the physician has good reason to believe that participation in the research study will not adversely affect the health of the patients who serve as research subjects.

15. Appropriate compensation and treatment for subjects who are harmed as a result of participating in research must be ensured.

Risks, Burdens and Benefits

16. In medical practice and in medical research, most interventions involve risks and burdens.

Medical research involving human subjects may only be conducted if the importance of the objective outweighs the risks and burdens to the research subjects.

17. All medical research involving human subjects must be preceded by careful assessment of predictable risks and burdens to the individuals and groups involved in the research in comparison with foreseeable benefits to them and to other individuals or groups affected by the condition under investigation.

Measures to minimise the risks must be implemented. The risks must be continuously monitored, assessed and documented by the researcher.

18. Physicians may not be involved in a research study involving human subjects unless they are confident that the risks have been adequately assessed and can be satisfactorily managed.

When the risks are found to outweigh the potential benefits or when there is conclusive proof of definitive outcomes, physicians must assess whether to continue, modify or immediately stop the study.

Vulnerable Groups and Individuals

19. Some groups and individuals are particularly vulnerable and may have an increased likelihood of being wronged or of incurring additional harm.

All vulnerable groups and individuals should receive specifically considered protection.

20. Medical research with a vulnerable group is only justified if the research is responsive to the health needs or priorities of this group and the research cannot be carried out in a non-vulnerable group. In addition, this group should stand to benefit from the knowledge, practices or interventions that result from the research.

Scientific Requirements and Research Protocols

21. Medical research involving human subjects must conform to generally accepted scientific principles, be based on a thorough knowledge of the scientific literature, other relevant sources of information, and adequate laboratory and, as appropriate, animal experimentation. The welfare of animals used for research must be respected.

22. The design and performance of each research study involving human subjects must be clearly described and justified in a research protocol.

The protocol should contain a statement of the ethical considerations involved and should indicate how the principles in this Declaration have been addressed. The protocol should include information regarding funding, sponsors, institutional affiliations, potential conflicts of interest, incentives for

subjects and information regarding provisions for treating and/or compensating subjects who are harmed as a consequence of participation in the research study.

In clinical trials, the protocol must also describe appropriate arrangements for post-trial provisions.

Research Ethics Committees

23. The research protocol must be submitted for consideration, comment, guidance and approval to the concerned research ethics committee before the study begins. This committee must be transparent in its functioning, must be independent of the researcher, the sponsor and any other undue influence and must be duly qualified. It must take into consideration the laws and regulations of the country or countries in which the research is to be performed as well as applicable international norms and standards but these must not be allowed to reduce or eliminate any of the protections for research subjects set forth in this Declaration.

The committee must have the right to monitor ongoing studies. The researcher must provide monitoring information to the committee, especially information about any serious adverse events. No amendment to the protocol may be made without consideration and approval by the committee. After the end of the study, the researchers must submit a final report to the committee containing a summary of the study's findings and conclusions.

Privacy and Confidentiality

24. Every precaution must be taken to protect the privacy of research subjects and the confidentiality of their personal information.

Informed Consent

25. Participation by individuals capable of giving informed consent as subjects in medical research must be voluntary. Although it may be appropriate to consult family members or community leaders, no individual capable of giving informed consent may be enrolled in a research study unless he or she freely agrees.

26. In medical research involving human subjects capable of giving informed consent, each potential subject must be adequately informed of the aims, methods, sources of funding, any possible conflicts

of interest, institutional affiliations of the researcher, the anticipated benefits and potential risks of the study and the discomfort it may entail, post-study provisions and any other relevant aspects of the study. The potential subject must be informed of the right to refuse to participate in the study or to withdraw consent to participate at any time without reprisal. Special attention should be given to the specific information needs of individual potential subjects as well as to the methods used to deliver the information.

After ensuring that the potential subject has understood the information, the physician or another appropriately qualified individual must then seek the potential subject's freely-given informed consent, preferably in writing. If the consent cannot be expressed in writing, the non-written consent must be formally documented and witnessed.

All medical research subjects should be given the option of being informed about the general outcome and results of the study.

27. When seeking informed consent for participation in a research study the physician must be particularly cautious if the potential subject is in a dependent relationship with the physician or may consent under duress. In such situations the informed consent must be sought by an appropriately qualified individual who is completely independent of this relationship.

28. For a potential research subject who is incapable of giving informed consent, the physician must seek informed consent from the legally authorised representative. These individuals must not be included in a research study that has no likelihood of benefit for them unless it is intended to promote the health of the group represented by the potential subject, the research cannot instead be performed with persons capable of providing informed consent, and the research entails only minimal risk and minimal burden.

29. When a potential research subject who is deemed incapable of giving informed consent is able to give assent to decisions about participation in research, the physician must seek that assent in addition to the consent of the legally authorised representative. The potential subject's dissent should be respected.

30. Research involving subjects who are physically or mentally incapable of giving consent, for example, unconscious patients, may be done only if the physical or mental condition that prevents

giving informed consent is a necessary characteristic of the research group. In such circumstances the physician must seek informed consent from the legally authorised representative. If no such representative is available and if the research cannot be delayed, the study may proceed without informed consent provided that the specific reasons for involving subjects with a condition that renders them unable to give informed consent have been stated in the research protocol and the study has been approved by a research ethics committee. Consent to remain in the research must be obtained as soon as possible from the subject or a legally authorised representative.

31. The physician must fully inform the patient which aspects of their care are related to the research. The refusal of a patient to participate in a study or the patient's decision to withdraw from the study must never adversely affect the patient-physician relationship.

32. For medical research using identifiable human material or data, such as research on material or data contained in biobanks or similar repositories, physicians must seek informed consent for its collection, storage and/or reuse. There may be exceptional situations where consent would be impossible or impracticable to obtain for such research. In such situations the research may be done only after consideration and approval of a research ethics committee.

Use of Placebo

33. The benefits, risks, burdens and effectiveness of a new intervention must be tested against those of the best proven intervention(s), except in the following circumstances:

Where no proven intervention exists, the use of placebo, or no intervention, is acceptable; or

Where for compelling and scientifically sound methodological reasons the use of any intervention less effective than the best proven one, the use of placebo, or no intervention is necessary to determine the efficacy or safety of an intervention

and the patients who receive any intervention less effective than the best proven one, placebo, or no intervention will not be subject to additional risks of serious or irreversible harm as a result of not receiving the best proven intervention.

Extreme care must be taken to avoid abuse of this option.

Post-Trial Provisions

34. In advance of a clinical trial, sponsors, researchers and host country governments should make provisions for post-trial access for all participants who still need an intervention identified as beneficial in the trial. This information must also be disclosed to participants during the informed consent process.

Research Registration and Publication and Dissemination of Results

35. Every research study involving human subjects must be registered in a publicly accessible database before recruitment of the first subject.

36. Researchers, authors, sponsors, editors and publishers all have ethical obligations with regard to the publication and dissemination of the results of research. Researchers have a duty to make publicly available the results of their research on human subjects and are accountable for the completeness and accuracy of their reports. All parties should adhere to accepted guidelines for ethical reporting. Negative and inconclusive as well as positive results must be published or otherwise made publicly available. Sources of funding, institutional affiliations and conflicts of interest must be declared in the publication. Reports of research not in accordance with the principles of this Declaration should not be accepted for publication.

Unproven Interventions in Clinical Practice

37. In the treatment of an individual patient, where proven interventions do not exist or other known interventions have been ineffective, the physician, after seeking expert advice, with informed consent from the patient or a legally authorised representative, may use an unproven intervention if in the physician’s judgement it offers hope of saving life, re-establishing health or alleviating suffering. This intervention should subsequently be made the object of research, designed to evaluate its safety and efficacy. In all cases, new information must be recorded and, where appropriate, made publicly available.

APPENDIX 2 - Centralization Of Peripheral Blood And Bone Marrow Samples

Hospital Universitario de Salamanca (HUSA), Centro de Investigación Médica Aplicada (CIMA) (Pamplona) and *Hospital Universitario 12 de Octubre (Madrid)* have been designated as the reference laboratories.

1. AT DIAGNOSIS

Bone marrow

A bone marrow (BM) sample should be collected at diagnosis (at least the following quantities):

- 10 cc EDTA: this volume will be used for:
  - 1 cc: DNA extraction. Sufficient volume at diagnosis.
  - 9 cc: for flow cytometry tests (MFC) and cell separation using a cell classifier.
  - Extraction of an additional 10ml is also recommended in case of damage to any sample.
- 10 cc heparin: this volume will be used in the separation process with AutoMacs®, before fluorescence in-situ hybridization studies (FISH). The separated sample will be frozen for genetic testing with purified plasma cells.

To ensure the BM samples are valid, they should be extracted from different areas in order to prevent contamination with medullary blood.

Peripheral blood (PB)

Peripheral blood samples will be collected from all patients at the screening visit:

- 10 cc EDTA, (x2):
  - 10 ml for analysis using MFC to identify, quantify, and separate circulating plasma cells. Other possible clonal cells that are identified will also be collected.
  - 10 ml for genetic testing. The plasma will be separated from the cells and frozen for the purpose of extracting nucleic acid and for genetic analyses. The cells will be used to extract DNA from germ cells.
- 10 cc serum (gel or PS tube, no additive):
  - Hevylite® and other protein assays (QIP-Mass spectrometry). Serum to be stored at -20ºC

at reference laboratories.

2. SAMPLES FOR MONITORING

Bone marrow

Bone marrow will need to be collected at any time the patient achieves complete remission, as long as this does not coincide with the specific times at which a bone marrow collection is already planned. In addition to the screening visit, these times are:

- At months 6, 12, 18, 24, and yearly after achieving complete remission.
- At the time of suspected CR.
- At the time of disease progression.
- Optional: at EOT if PD MRd-ve rate will be reported as the % of patients in MRd-ve.

During these specific times during follow-up a tube of 10ml EDTA will be sent and distributed in the central laboratory as follows:

- 10 ml EDTA, to evaluate the presence of minimal residual disease (MRD) using MFC.

Peripheral blood

The following is required:

- 10 ml (x2) EDTA, to evaluate the presence of MRD in the plasma and cells via MFC.
- 10 ml tube, no additives (serum) for protein assays (HevyLite® and QIP-Mass spectrometry).

These samples will be collected:

- At months 6,12, 18, 24, and yearly after achieving complete remission.
- At the time of disease progression.
- At any time, patient achieves complete remission.

3. SAMPLES WHEN RELAPSE/PROGRESSION OCCURS

All procedures will be exactly the same as those carried out at the time of diagnosis.

If extramedullary disease is observed, collection of one disease sample (via aspiration or biopsy) which is placed in normal saline with EDTA is recommended.

FLOW CYTOMETRY

Evaluation of minimal residual disease by flow cytometry (MRD-MFC) will consist of a relatively rapid, highly sensitive, reproducible biomarker to detect the presence of phenotypically abnormal clonal plasma cells (MRD). The methodology that will be used to process large volumes of samples (to ensure a high degree of sensitivity) is a bulk lysis-based approach, before performing conventional sorting of activated cells by fluorescence (FACS).

Bone marrow cells will be stained with two combinations of 8 monoclonal antibody colors:

Tube	V450	BV510	FITC	PE	PerCPCy5.5	PECy7	APC	APCC750
1	CD138	CD27	CD38	CD56	CD45	CD19	CD117	CD81
2	CD138	CD27	CD38	CD56	CD45	CD19	cyKAPPA	cyLAMBDA

The strategy based on the use of two tubes will include as a last resort monitoring of myeloma cells using 10-color flow cytometry. This will permit unique, high specificity confirmation of the monoclonality of phenotypically abnormal plasma cells (by light chain restriction). Said cells will have been clearly identified by low antigen expression (CD19, CD27, CD38, CD45, CD81) or overexpression (CD56, CD117, CD138). This strategy is possible thanks to the merge and calculation options available using *Infinicyt* software for analyses of flow data, as described below:

The presence of surface markers (CD138, CD27, CD38, CD56, CD45, CD19) and certain morphological characteristics (FSC and SSC) permit the specific identification of plasma cells (PC). The merge function in Infinicyt™ software will then be used to combine different data files corresponding to the 2 different aliquots of the same sample. This will create a new combined data file (unique file) that will contain all of the information measured in the same sample, including the antigens that have been measured directly and those not evaluated directly (missing values) for an individual cell in a specific aliquot of the sample (for example CD117 and CD81 in tube 2). The “Calculation” function in the Infinicyt™ software will then be used to estimate the “missing values”.

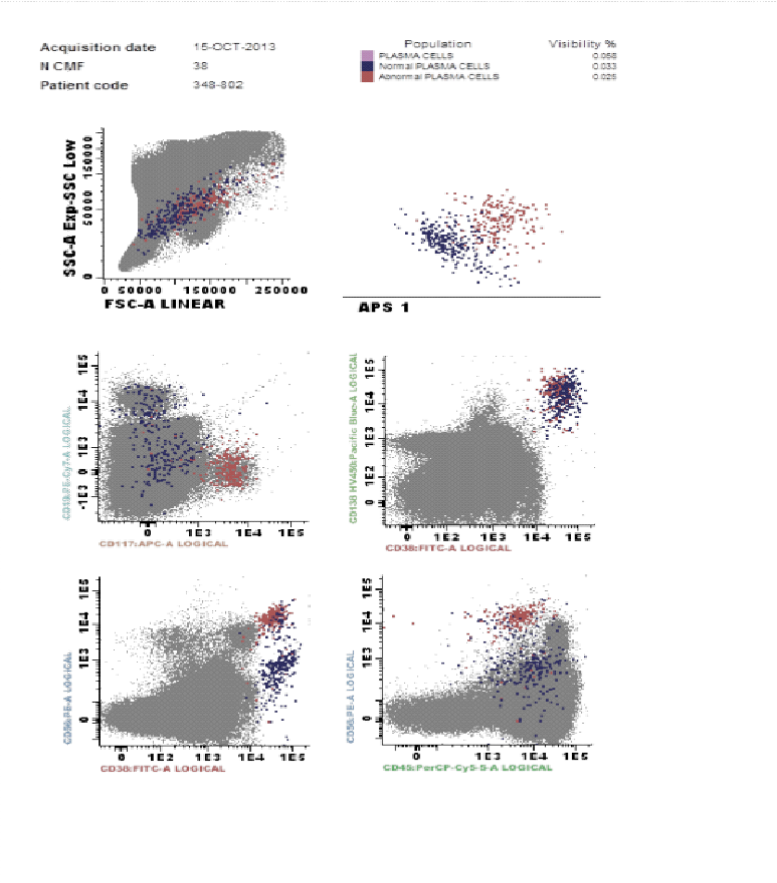
To do this, the software searches for the “nearest” event in each of the other aliquots from the sample, based simply on its unique position in the multidimensional space created by all of the common basic parameters. In other words, for each unique clonal plasma cell in a patient with recently-diagnosed symptomatic MM, 10 parameters will be evaluated which will be integrated into

one representative vector for each patient via an automated analysis of the main component, based on a pattern.

This unique strategy will guarantee highly sensitive detection of minimal residual disease (10^{-5}) by flow cytometry, even if the information about the diagnostic sample is not available.

The report for each patient will include dot plots and graphic representations of the analysis of the main component, which will be available to the patient's referring physician and /or the company.

The project manager will analyze the sample, include all results in the analytical database and generate a graphical report of the sample analysis. In all samples in which MRD has been detected (below 10^{-5} cells) a second project manager will perform a blind analysis of the sample in order to determine the accuracy and reproducibility of the results. The graphical report of the analysis will be printed only if the company requests it, and it will be stored electronically by the laboratory. The graphical report will also include the laboratory serial number and the patient code for the purpose of ensuring the reliability of the chain of custody.



PET/CT

Imaging MRD will be monitored at screening, after induction treatment and in patients in CR, according to the availability of PET/CT in the respective hospitals and to the decisions made by the imaging-working group.

APPENDIX 3 - NCI-CTCAE-CRITERIA

Common terminology criteria for classification of adverse events version 5.0 (NCI CTC AEv5) are available at the following Internet address:

https://ctep.cancer.gov/protocolDevelopment/electronic_applications/ctc.htm

(National Cancer Institute 2021)

APPENDIX 4 - ECOG AND KARNOFSKY PERFORMANCE STATUS

Karnofsky and ECOG performance status scales are shown in the table below:

Karnofsky	Description	ECOG
100	Normal, no complaints; no evidence of disease.	0
90	Able to carry on normal activity. Minor signs or symptoms of disease.	0
80	Normal activity with effort; some signs or symptoms of disease	1
70	Cares for self; unable to carry on normal activity or to do active work.	1
60	Requires occasional assistance, but is able to care for most of his/her personal needs	2
50	Requires considerable assistance and frequent medical care.	2
40	Disabled; requires special care and assistance.	3
30	Severely disabled; admission to hospital is indicated although death not imminent.	3
20	Very sick; hospital admission necessary; active supportive therapy required.	4
10	Moribund; fatal processes progressing rapidly.	4
0	Dead	5

*Karnofsky D, Burchenal J, The clinical evaluation of chemotherapeutic agents in cancer. In: MacLeod C, ed. Evaluation of Chemotherapeutic Agents. New York, NY: Columbia University Press; 1949:191–205.

**Zubrod C, et al. Appraisal of methods for the study of chemotherapy in man: Comparative therapeutic trial of nitrogen mustard and thiophosphoramide. *Journal of Chronic Diseases*; 1960:11:7-33.

Available at: http://ecog-acrin.org/resources/ecog-performance-status

APPENDIX 5 - Response Criteria

Response will be assessed using IMWG criteria:

Response	Response criteria[1]
CR with sustained negative MRD	CR with MRD negative status in BM (by flow cytometry or molecular testing) as defined below, confirmed with one year of follow-up.
Molecular complete remission (mCR)	CR and absence of clonal plasma cells in BM analyzed using large-scale sequencing techniques with a minimum sensitivity of 10^{-5} (1 cell in 10^5 nucleated cells).
Immunophenotypic complete remission (CR – flow cytometry)	RC and absence of phenotypically abnormal clonal plasma cells in BM, using EuroFlow standard analysis recommendations to detect MRD in MM (minimum sensitivity of 1 cell in 10^5 nucleated cells).
Stringent complete response (sCR)	The following criteria must be met: 1. Disappearance of monoclonal protein in blood and urine, confirmed in immunofixation studies 2. Disappearance of soft tissue plasmacytomas. 3. BMPC < 5%.[2] 4. Normal serum kappa/lambda ratio in sFLC determinations. 5. Absence of clonal cells in BM[3], as shown by immunohistochemistry or immunofluorescence.
Complete Response (CR)	1. Disappearance of monoclonal protein in blood and urine confirmed by immunofixation 2. Disappearance of soft tissue plasmacytomas for at least 6 weeks. 3. < 5% BMPC[2]
Very good partial response (VGPR)	1. Disappearance of serum and urine monoclonal protein on electrophoresis even if detectable by immunofixation 2. ≥ 90% reduction in serum monoclonal protein and less than 100 mg of monoclonal protein detected in 24-hr urine sample
Partial response (PR)	• ≥ 50% reduction in serum monoclonal protein and ≥ 90% reduction in urine monoclonal protein, or < 200 mg of monoclonal protein excreted in 24hr urine sample • If the serum and urine M-protein are not measurable, a decrease of ≥ 50% in the difference between involved and uninvolved FLC levels is required in place of the M-protein criteria. • If serum and urine M-protein are not measurable, and serum free light assay is also not measurable, ≥ 50% reduction in bone marrow PCs is required in place of M-protein, provided baseline bone marrow plasma cell percentage was ≥ 30%. • In addition to the above criteria, if present at baseline, a ≥ 50% reduction in the size of soft tissue plasmacytomas is also required.
Stable disease (SD)	Not meeting criteria for sCR, CR, VGPR or disease progression
Progressive disease	One or more of the following criteria must be met:

(PD)	- Increase of 25% from lowest confirmed response value in one or more of the following criteria: ✔ ≥ 25 % increase in serum concentration of monoclonal paraprotein (absolute increase must be at least 5 g/L). ✔ ≥ 25 % increase in light chain excretion in 24hr urine sample (with an absolute increase of at least 200 mg / 24 hours). ✔ For patients without measurable disease in serum and/or urine: absolute increase >10% in the difference in concentration of involved and uninvolved free light chains. ✔ In patients without measurable serum and urine M-protein levels, and without measurable involved FLC levels, bone marrow plasma-cell percentage irrespective of baseline status (absolute increase must be ≥10%). - Development of new soft tissue plasmacytomas or bone lesions (osteoporotic fractures do not constitute progression) or definite increase in the size of existing plasmacytomas or bone lesions. A definite increase is defined as a 50% (and ≥1 cm) increase of soft tissue plasmacytomas. - Appearance of hypercalcemia (corrected serum calcium > 11.5 mg/dL) that can be attributed exclusively to myeloma.

1 Two consecutive measurements are required for all response categories. All categories require the absence of disease progression and no new bone lesions. These response criteria do not require routine radiological tests.
2 It is not necessary to repeat the bone biopsy to confirm the absence of plasma cells in BM.
3 In all categories related to relapse or progression, two consecutive measurements are required before disease relapse or progression is determined.

APPENDIX 6- ALGORITHMS USED IN THIS PROTOCOL

- **Body surface area:**

Body surface area must be calculated using a standard nomogram. Following it can be found an example of normogram:

$$\text{BSA } (m^2) = \sqrt{\frac{Height\ (cm) x\ Weight\ (kg)}{3{,}600}}$$

- **Corrected calcium:**

It will be calculated using the following formula and only for those patients in which during the screening visit albumin was out of range.

If calcium is expressed in mg/dL and albumin is expressed in g/dL:

Corrected calcium (mg/dL) = serum calcium (mg/dL) + 0.8 x (4 – serum albumin [g/dL])

If calcium is expressed in mmol/L and albumin is expressed in g/L:

Corrected calcium (mmol/L) = serum calcium (mmol/L) + 0.02 x (40 – serum albumin [g/L])

Source: Burtis 1998.

- **Formula for Estimating Glomerular Filtration Rate using Modified Diet in Renal Disease (MDRD) Equation:**

For standardized serum creatinine in **mg/dL**, the estimated GFR for the MDRD formulas is:

$$\text{eGFR mL/min} = 175 \times [\text{standardized serum creatinine (mg/dL)}]^{-1.154} \times [\text{age}]^{-0.203} \times [1.212_{\text{if Black}}] \times [0.742_{\text{if female}}] \times \text{BSA}^* / 1.73$$

Creatinine levels conversion from µmol/L to mg/dL:

$$\text{creatinine (mg/dL)} = \frac{\text{creatinine (µmol/L)}}{88.4}$$

APPENDIX 7 - INFORMED CONSENT FORM

Included in a separate document

APPENDIX 8– DOCUMENT OF REPORTING OF SERIOUS ADVERSE EVENTS

Included in a separate document

APPENDIX 9- Document Of Pregnancy Reporting

Study Code:	**GEM-RANTAB**		**Study Site:**	
Subject No:		**Subject Initials:**		
Country:				
Investigational Product(s)				
Therapy start date: (yy-mmm-dd)		Therapy stop date: (yy-mmm-dd)		
Date of last menstruation: (yy-mmm-dd)		Date of expected delivery: (yy-mmm-dd)		
Investigator's signature:		Date: (yy-mmm-dd)		
Keep the original of this form in the Investigator's File and a copy in the CRF. Please e-mail to XXXXXXXX@XXXXX, or by fax at: +34				
Infant Follow up				
Pregnancy result (mark with X)				
Live birth	Elective abortion	Spontaneous abortion	Stillbirth	Unknown
Please fill in a SAE form in case of any spontaneous abortion, stillbirth, birth defect/congenital anomaly, death, or other serious infant condition.				
Infant delivery date (yy-mmm-dd)		Condition of infant		
Normal		Abnormal*		
* If abnormal, please comment				
Comments:				
Investigator's signature:		Date: (yy-mmm-dd)		
Printed name				

APPENDIX 10 - Correlative Studies

Three reference laboratories have been established (Hospital Universitario de Salamanca, Hospital 12 de Octubre de Madrid and Centro de Investigación Médica Aplicada (CIMA) of Pamplona). This number may still increase depending on how enrollment develops.

Distribution. Participating sites will be distributed between the three reference laboratories. Each site will be informed about the distribution details during the initial visit and by means of the Sample Management documentation that will be provided by the trial Clinical Coordinators and filed at each site.

Samples. 10cc of bone marrow in EDTA and 10cc in lithium heparin (flow cytometry, cytogenetic and molecular biology), 10cc of serum in gelose (Heavylite and Freelite and storage of serum for correlative studies), and 20cc of peripheral blood in EDTA (flow cytometry and molecular biology). Samples will be shipped at room temperature in less than 24 hours after collection.

Identification of samples. Patient identification number, Protocol code, time of disease (screening, at months 6, 12, 18, 24, end-of-study visit and at any time to confirm complete response), shipping hospital and name and telephone number of contact physician.

Time of shipping. (see APPENDIX 2)

Transportation and contacts. The appropriate containers for the trial will be sent by the Sponsor to the list of participating hospitals. The shipment of the containers must always be requested in advance by the participating hospitals by email to investigacion@mfar.net

Business hours. Samples will be accepted during arrival times from Monday to Thursday, and until 10:00 am on Friday (this means that **samples must be taken between Monday and Thursday, never on Friday, Saturday or Sunday)**. If a sample is collected on Friday, it must be stored at site at the corresponding temperature and shipment to be performed on Monday. Please arrange for your samples so that they will arrive before 10:00 am (at each destination site, check with the courier). You must call to arrange delivery outside of these hours. (Please take into account the local holidays at the place of destination)

The PETHEMA Foundation will cover the cost of transportation, using a courier company**.** To request transport, the operator will need a subscriber number and the Protocol code for this trial.

The PETHEMA Foundation will cover the cost of transportation, using a courier company. Further information is provided in the study lab manual.

Samples will be sent to:

Salamanca:

Attn: Noemi Puig / Norma Gutiérrez / Ramón García Sanz.
Hospital Universitario de Salamanca.
Unidad de Inmunopatología/Biología Molecular/Citogenética.
Paseo de la Transición Española, Bloque I, Planta 3, 426.
37007 Salamanca.
Tel: 923 291100 Ext. 56616, 56620, 55606
Email: rgarcias@usal.es, normagu@usal.es, noepuig@gmail.com

Pamplona:

Attn: Bruno Paiva / M.ª José Calasanz.
CIMA LAB Diagnostics
Laboratorio de Citometría de Flujo (1.07). Centro de Investigación Médica Aplicada (CIMA)
Av. Pio XII 55, 31008 Pamplona.
Tel: 948 194 700. Ext: 1038. / Fax: 948 194 714.
Email: bpaiva@unav.es / ivurio@unav.es

Madrid:

Attn: Dr. Joaquín Martínez López / Dra. Teresa Cedena
Unidad de Biología Molecular.
Servicio de Hematología. Hospital Universitario 12 de Octubre.
Centro de Actividades Ambulatorias, 6ª planta bloque D.
Avda. de Córdoba s/n, 28041 Madrid
Tel: 91 779 27 87 / 91 779 23 03.
Email: jmarti01@med.ucm.es / mariateresa.cedena@salud.madrid.org

More details about sample management can be found in appendix 2 and in the study Lab Manual.

APPENDIX 11. Subcutaneous Injection Site Locations

Injection site locations include a maximum of 4 unique administration sites distributed across the 2 lower and the 2 upper abdominal quadrants (up to 1 injection location per quadrant).

Administer the required number of injections in the following order:

1. Lower left quadrant
2. Lower right quadrant
3. Upper left quadrant
4. Upper right quadrant

Injections to the abdomen are preferred. If SC injections in the abdominal location are not possible, SC injections can be administered in a distributed manner in the thighs. SC injections in the upper extremities (eg, deltoid, upper and lower arm) are not permitted

Track the participant's injection site(s) sequentially on this diagram with a red pen and mark the injection sites on the participant's abdomen according to your clinic's standard practice.

Record the location, time of each injection and any injection site reactions in the participant's source records, and in the study CRF. Complete one CRF per injection.

APPENDIX 12. CRS AND ICANS GRADING, MITIGATION, AND MANAGEMENT

CYTOKINE RELEASE SYNDROME

CRS is a non-antigen-specific cytokine-associated toxicity that occurs as a result of

high-level immune activation. CRS is a potentially life-threatening toxicity that has been observed following administration of immune-base therapies for cancer (antibodies and adoptive T-cell therapies). CRS is likely to be a common toxicity that can be managed through supportive care and anti-cytokine interventions.

In cases of suspected CRS, a serum sample should be provided for cytokine release assay analysis by the local lab as long as the sampling does not interfere with the medical treatment of the participant. If CRS is suspected, additional blood samples should also be collected for central cytokine analysis if not already scheduled.

Early intervention should be undertaken at the first sign of CRS; signs may include pyrexia, tachycardia, tachypnea and/or hypotension, and are temporally related to elranatamab in the absence of alternative etiologies.

CRS grading will follow ASTCT criteria (Table 15) (Lee et al, 2019). For CRS management, published treatment guidelines are recommended, but they may be modified as needed by the responsible investigator according to the best practices at their institute (Neelapu et al, 2018; Neelapu, 2019).

Table 7. ASTCT CRS Grading

CRS parameter:	**Fever[a]**	**With Hypotension**	**And/or[b] Hypoxia**
Grade 1	Temp ≥38°C	None	None
Grade 2		Not requiring vasopressors	Requiring low-flow[c] nasal cannula, low-flow[c] facemask or blow-by
Grade 3		Requiring a vasopressor with or without vasopressin	Requiring high-flow[c] nasal cannula, high-flow[c] facemask, nonrebreather mask, or Venturi mask

Grade 4		Requiring multiple vasopressors (excluding vasopressin)	Requiring positive pressure (eg, CPAP, BiPAP, intubation and mechanical ventilation)

Note: Organ toxicities associated with CRS should be graded according to CTCAE v5.0 and do not influence CRS grading.

a Fever: Temp ≥38°C and not attributable to any other cause. In participants who have CRS then receive antipyretic or anti-cytokine therapy such as tocilizumab or steroids, fever is no longer required to grade subsequent CRS severity. In this case, CRS grading is driven by hypotension and/or hypoxia.

b CRS grade is determined by the more severe event: hypotension or hypoxia not attributable to any other cause. For example, a participant with Temp of 39.5°C, hypotension requiring 1 vasopressor, and hypoxia requiring low-flow nasal cannula is classified as Grade 3 CRS.

c Low-flow nasal cannula or facemask is defined as oxygen delivered at ≤6 L/min. Low flow also includes blow-by oxygen delivery, sometimes used in pediatrics. High-flow nasal cannula or facemask is defined as oxygen delivered at >6 L/min. This is modified from original ASTCT criteria to differentiate between low- flow and high-flow facemask.

Source: (Lee et al, 2019)

CRS management guidelines by ASTCT Severity Grading (Neelapu et al, 2018; Neelapu, 2019)

Fever, regardless of grade of CRS, is managed as described under Grade 1 CRS.

- **Grade 1 CRS:**

Fever

- Acetaminophen/paracetamol and hypothermia blanket for the treatment of fever.
- NSAIDs such as ibuprofen can be used as second treatment option for fever if not contraindicated.
- Assess for infection using blood and urine cultures, and chest radiography.
- Empiric broad-spectrum antibiotics and filgrastim if neutropenic.
- Maintenance IV fluids for hydration.
- Symptomatic management of constitutional symptoms or organ toxicity.
- Consider tocilizumab 8 mg/kg* IV or siltuximab 11 mg/kg IV for persistent (lasting >3 days) and refractory fever.

- **Grade 2 CRS:**

Monitor vital signs every 4 hours, minimally, for worsening of condition.

- **Hypotension**
- IV fluid bolus of 500-1000 ml of normal saline. Consider giving a second fluid bolus if systolic BP remains <90 mmHg.
- Consider tocilizumab 8 mg/kg (maximum dose 800 mg) IV or siltuximab

11 mg/kg IV for treatment of hypotension refractory to fluid boluses; tocilizumab can be repeated after 6 hours if needed.

- If hypotension persists after 2 fluid boluses and anti-IL-6 therapy, start vasopressors, consider transfer to ICU, obtain ECHO, and initiate other methods of hemodynamic monitoring.

- In participants at high-risk (bulky disease, older age and/or comorbidities) or if hypotension persists after 1-2 doses of anti-IL-6 therapy, dexamethasone can be used at 10 mg IV every 6 hours.
- **Hypoxia**
- Supplemental oxygen.

Tocilizumab or siltuximab ± corticosteroids and supportive care, as indicated for hypotension.

- **Grade 3 CRS:**

Monitor participant (including continuous ECG monitoring) in an ICU and obtain ECHO if not done already.

- **Hypotension**
- IV boluses, as needed, as recommended for Grade 2 CRS.
- Tocilizumab and siltuximab as recommended for Grade 2 CRS if not administered previously.
- Vasopressors as needed.
- Dexamethasone 10 mg IV every 6 hours; if refractory, increase to 20 mg IV every 6 hours.
- **Hypoxia**
- Supplemental oxygen including high-flow oxygen delivery.
- Tocilizumab or siltuximab plus corticosteroids and supportive care, as described above for Grade 2 CRS.

- **Grade 4 CRS:**

Monitor participant (including continuous ECG monitoring) in an ICU and obtain ECHO if not done already.

- **Hypotension**
- IV boluses, anti-IL-6 therapy, vasopressors, and hemodynamic monitoring as recommended for Grade 3 CRS.

- Methylprednisolone 1 g/day IV.
- **Hypoxia**
- Supplemental oxygen via positive pressure/mechanical ventilation.
- Tocilizumab or siltuximab plus corticosteroids and supportive care, as described above for Grade 2 CRS.

Immune effector cell-associated neurotoxicity syndrome

Although less commonly seen than CRS, ICANS has been observed with some T-cell directed therapies and may manifest as aphasia, delirium, encephalopathy, lethargy, difficulty concentrating, agitation, tremor, seizures, and cerebral edema (Lee et al, 2019). If ICANS is observed in relation to elranatamab, the ASTCT criteria will be used for grading and published guidelines are recommended for management (Neelapu et al, 2018; Lee et al, 2019; Neelapu, 2019). These treatment guidelines may be modified as needed by the responsible investigator according to the best practices at their institute.

Table 8. Immune Effector Cell-Associated Encephalopathy (ICE) Score

Category	Task	Points
Orientation	Orientation to year, month, city, hospital	4
Naming	Ability to name 3 objects	3
Following commands	Ability to follow simple commands	1
Writing	Ability to write a standard sentence	1
Attention	Ability to count backwards from 100 by 10	1

Table 9. ASTCT ICANS Grading

Neurotoxicity Domain	Grade 1	Grade 2	Grade 3	Grade 4
ICE score[a]	7-9	3-6	0-2	0 (unarousable and unable to perform ICE)
Depressed level of consciousness[b]	Awakens spontaneously	Awakens to voice	Awakens only to tactile stimulus	Unarousable or requires vigorous or repetitive tactile stimuli to arouse. Stupor or coma
Seizure	N/A	N/A	Any clinical seizure that resolves rapidly or non-convulsive seizures on EEG that resolve with intervention	Life-threatening prolonged seizure (>5 min); or repetitive clinical or electrical seizures without return to baseline in between
Motor findings[c]	N/A	N/A	N/A	Deep focal motor weakness such as hemiparesis or paraparesis
Elevated ICP/cerebral edema	N/A	N/A	Focal/local edema on neuroimaging[d]	Diffuse cerebral edema on neuroimaging; decerebrate or decorticate posturing; or cranial nerve VI (abducens nerve) palsy; or papilledema; or Cushing's triad

Note: ICANS grade is determined by the most severe event (ICE score, level of consciousness, seizure, motor findings, raised ICP/cerebral edema) not attributable to any other cause; for example, a participant with an ICE score of 3 who has a generalized seizure is classified as Grade 3 ICANS.

[a] A participant with an ICE score of 0 may be classified as Grade 3 ICANS if awake with global aphasia, but a participant with an ICE score of 0 may be classified as Grade 4 ICANS if unarousable.

[b] Depressed level of consciousness should be attributable to no other cause (eg, no sedating medication).

[c] Tremors and myoclonus associated with immune effector cell therapies may be graded according to CTCAE v5.0; these symptoms do not influence ICANS grading.

[d] Intracranial hemorrhage with or without associated edema is not considered a neurotoxicity feature and is excluded from ICANS grading. It should be graded according to CTCAE v5.0.

Source: (Lee et al, 2019)

ICANS Management Guidelines Per ASTCT (Neelapu et al, 2018**;** Neelapu, 2019**)**

- **ICANS Grade 1:**
- Vigilant supportive care; aspiration precautions; IV hydration.
- Withhold oral intake of food, medicines, and fluids; assess swallowing.
- Convert all oral medications and/or nutrition to IV if swallowing is impaired.
- Avoid medications that cause CNS depression.
- Neurology consultation.
- If suspected, evaluate for elevated ICP with fundoscopic exam for papilledema and lumbar puncture for CSF opening pressure.
- MRI of the brain with and without contrast; CT scan of the brain can be performed if MRI is not feasible.
- Daily 30 min EEG until symptoms resolve.
- Consider anti-IL-6 therapy with tocilizumab 8 mg/kg (maximum 800 mg) IV or siltuximab 11 mg/kg IV in case of concurrent CRS.

- **ICANS Grade 2:**
- Supportive care and neurological work-up as described for Grade 1 ICANS.
- Anti-IL-6 therapy if associated with concurrent CRS, as described for Grade 1 ICANS and if not administered previously.
- Dexamethasone 10 mg IV every 6 hours or methylprednisolone 1 mg/kg IV every 12 hours if refractory to anti-IL-6 therapy, or for ICANS without concurrent CRS.
- Consider transferring participant to ICU if ICANS associated with Grade ≥2 CRS.

- **ICANS Grade 3:**
- Supportive care and neurological work-up as indicated for Grade 1 ICANS.

- ICU transfer is recommended.
- If EEG shows non-convulsive status epilepticus:
  - Assess airway, breathing, and circulation; check blood glucose.
  - Lorazepam 0.5 mg IV, with additional 0.5 mg IV every 5 min, as needed, up to a total of 2 mg to control electrographical seizures.
  - Levetiracetam 500 mg IV bolus, as well as maintenance doses.
  - If seizures persist, transfer to ICU and treat with phenobarbital loading dose of 60 mg IV.
  - Recommended maintenance therapy after resolution of non-convulsive status epilepticus are as follows:
    - lorazepam 0.5 mg IV every 8 hours for three doses;
    - levetiracetam 1,000 mg IV every 12 hours; duration of therapy per investigator/treating physician's discretion;
    - phenobarbital 30 mg IV every 12 hours; duration of therapy per investigator/treating physician's discretion.
  - Lacosamide may also be considered for treatment of seizures should the seizures persist. Lacosamide should not be used in participants with concurrent CRS in order to avoid arrhythmias and hypotension.
- For convulsive status epilepticus:
  - Assess airway, breathing, and circulation; check blood glucose.
  - Transfer to ICU.
  - Lorazepam 2 mg IV, with additional 2 mg IV to a total of 4 mg to control seizures.
  - Levetiracetam 500 mg IV bolus, as well as maintenance doses.
  - If seizures persist, add phenobarbital at a loading dose of 15 mg/kg IV.
  - Maintenance doses after resolution of convulsive status epilepticus:
    - lorazepam 0.5 mg IV every 8 hours for three doses;
    - levetiracetam 1,000 mg IV every 12 hours; duration of therapy per investigator/treating physician's discretion;

    - phenobarbital 1-3 mg/kg IV every 12 hours; duration of therapy per investigator/treating physician's discretion.
  - Lacosamide may also be considered for treatment of seizures should the seizures persist. Lacosamide should not be used in participants with concurrent CRS in order to avoid arrhythmias and hypotension.
  - Continuous EEG monitoring should be performed, if seizures are refractory to treatment.
- High-dose methylprednisolone IV 1 g/day for focal/local edema.
- Anti-IL-6 therapy if associated with concurrent CRS, as described for Grade 1 ICANS and if not administered previously.
- Corticosteroids as outlined for Grade 2 ICANS if symptoms worsen despite anti-IL-6 therapy, or for ICANS without concurrent CRS; continue corticosteroids until improvement to Grade 1 ICANS and then taper.

- **ICANS Grade 4:**
- Supportive care and neurological work-up as outlined for Grade 1 ICANS.
- ICU monitoring; consider mechanical ventilation for airway protection.
- Anti-IL-6 therapy and repeat neuroimaging as described for Grade 3 ICANS.
- High-dose corticosteroids continued until improvement to Grade 1 ICANS and then taper; for example, methylprednisolone IV 1 g/day for 3 days, followed by rapid taper at 250 mg every 12 hours for 2 days, 125 mg every 12 hours for 2 days, and 60 mg every 12 hours for 2 days.
- For convulsive status epilepticus, treat as described for Grade 3 ICANS.
- MRI of the spine should be obtained for focal motor weakness.
- To manage elevated ICP:
  - Elevate head of the participant's bed to an angle of 30 degrees.
  - Hyperventilation to achieve target PaCO2 of 28–30 mmHg, but maintained for no longer than 24 hours.

- Hyperosmolar therapy with either mannitol (20 g/dl solution) or hypertonic saline (3% or 23.4%, as detailed below):
  - Mannitol: initial dose 0.5–1 g/kg; maintenance at 0.25–1 g/kg every 6 hours while monitoring metabolic profile and serum osmolality every 6 hours and withhold mannitol if serum osmolality is ≥320 mOsm/kg, or the osmolality gap is ≥40.
  - Hypertonic saline: initial 250 ml of 3% hypertonic saline; maintenance at 50-75 ml/hour while monitoring electrolytes every 4 hours, and withhold infusion if serum Na levels reach ≥155 mEq/l.
  - For participants with imminent herniation: initial 30 ml of 23.4% hypertonic saline; repeat after 15 min, if needed.
  - If participant has Ommaya reservoir, drain CSF to target opening pressure of <20 mmHg
- Consider neurosurgery consultation for ventriculoperitoneal shunt in participants with cerebral edema, and IV anesthetics for burst-suppression pattern on EEG.
- Metabolic profiling every 6 hours and daily CT scan of head, with adjustments in usage of the aforementioned medications to prevent rebound cerebral oedema, renal failure, electrolyte abnormalities, hypovolemia, and hypotension.

APPENDIX 13. Birth Control Methods

Birth control methods which may be considered as highly effective (Suitable for this clinical trial)

For the purpose of this guidance, methods that can achieve a failure rate of less than 1% per year when used consistently and correctly are considered as highly effective birth control methods. Such methods include:

- Combined (estrogen and progestogen containing) hormonal contraception associated with inhibition of ovulation [1]:
  - oral
  - intravaginal
  - transdermal

- Progestogen-only hormonal contraception associated with inhibition of ovulation [1]:
  - oral
  - injectable
  - implantable [2]

- Intrauterine device (IUD) [2]

- Intrauterine hormone-releasing system (IUS) [2]

- Bilateral tubal occlusion [2]

- Vasectomised partner [2,3]

- Sexual abstinence [4]

1. Hormonal contraception may be susceptible to interaction with the IMP, which may reduce the efficacy of the contraception method
2. Contraception methods that in the context of this guidance are considered to have low user dependency.
3. Vasectomised partner is a highly effective birth control method provided that partner is the sole sexual partner of the WOCBP trial participant and that the vasectomised partner has received medical assessment of the surgical success.
4. In the context of this guidance sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study treatments. The reliability of sexual abstinence needs to be evaluated in relation to the duration of the clinical trial and the preferred and usual lifestyle of the patient.

Birth control methods which may not be considered as highly effective (not suitable for this clinical trial)

Acceptable birth control methods that result in a failure rate of more than 1% per year and should not be considered for this clinical trial include:

- Progestogen-only oral hormonal contraception, where inhibition of ovulation is not the primary mode of action
- Male or female condom with or without spermicide [5]
- Cap, diaphragm or sponge with spermicide [5]

5. A combination of male condom with either cap, diaphragm or sponge with spermicide (double barrier methods) are also considered acceptable, but not highly effective, birth control methods

APPENDIX 14. DATA SAFETY MONITORING COMMITTEE (DSMC)

OBJECTIVE OF THE COMMITTEE

The main objective of this committee is to safeguard the security of patients participating in the clinical trial by independent review of:

- Evaluation of the rates of mortality and morbidity and their possible relationship to the study drug
- Review of adverse events of especial interest (AESI) that have developed during the study.
- Supervision of pharmacovigilance procedures and, more specifically, Grade 3 and 4 Adverse Events.
- Review of Serious Adverse Events occurring during the study.
- Evaluation of overall ethical aspects over the course of the research study

COMMITTEE MEMBERS

The DSMC will be designated by the sponsor and composed by independent experts in the evaluation of patients with MM.

POPULATION TO BE REVIEWED

Information on patients that from the time of the preliminary evaluation have been recruited to date will be reviewed in terms of safety.

SOURCE DOCUMENTS TO BE REVIEWED

- Electronic Case Report Form (e-CRF) containing the required information.
- List of Serious Adverse Events (SAE).
- Members of the Committee can request more information or clarification of any kind directly from any of the investigators participating in the study.
- The Sponsor (Fundación PETHEMA), Study Coordinators, and the CRO in charge of the study (MFAR Clinical Research) will appropriately support the DSMC members providing them with any document/information to evaluate the objectives already described.

PROCEDURES

1. In-person Meetings

The objectives of these meetings will be:

- Review of the study parameters.
- Information on each patient or information from different patients will be reviewed, based on the decision of the DSMC members
- After the review, the DSMC will validate all patient data, or will send a query, or will request a report to be issued by the investigator.
- The duration of the meeting will depend on the number of patients to be reviewed.
- Meetings will be held at least once per year or more if the DSMC deems it necessary. If meetings cannot be held in person, teleconferences will be held instead and will be established on an ad hoc basis as patients are recruited.

2. Meeting Minutes or Summary

- After each meeting, minutes or a summary will be written in order to document the decisions made at each meeting.
- The minutes or summary will be approved and signed by all IDMC members present at the meeting.

3. Final Report

A final report with the results obtained will be issued after the review:

- Number of patients reviewed
- Findings
- Reconciliation of the DSMC's review and the Investigators' evaluations.